THE CHILD WITH MULTIPLE BRUISES: CHILD ABUSE

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Outline

- Principles of child protection
- Definition and types of child abuse
- Risk factors and long term effect of child abuse
- The injuries : intentional Vs non-intentional
- Management: local settings
- Prevention

Principles of child protection

- All children have the right to be protected against harm and exploitation
- The safety, needs, welfare and rights of the children should always come first

Procedures for Handling Child Abuse (Revised 2015), Social Welfare Department, HKSAR

Definition

Child abuse is defined as

- **any act of commission or omission that endangers or impairs the physical /psychological health and development of an individual under the age of 18**
  - Such act is judged on the basis of a combination of community standards and professional expertise

Procedures for Handling Child Abuse (Revised 2015), Social Welfare Department, HKSAR

Types of abuse

- Physical abuse
- Sexual abuse
- Neglect
- Psychological abuse
- Mixed/ Multiple abuses

Risk factors for child abuse

Family

- Social isolation
- Crisis or tension in family e.g. pregnancy, divorce/ separation, in-law conflict
- Cultural / superstitious beliefs
- Domestic violence e.g. spouse battering

Parents

- History of childhood abuse, experience of domestic/other violence
- History of psychiatric disorder
- Alcoholism / Drug abuse / Gambling
- Rigid or unreasonable expectation on the child
- Strong belief in corporal punishment
- Immature parents
- Poor impulse/ anger control
- Poor parenting skills

Child

- Unwanted child
- Illegitimate child
- Baby with feeding or sleeping problem
- Early separation from parents
- Child exposed to conflicting child care rearing practices e.g. child reared away from home
- Child with physical or mental disability
- Child associated with family misfortune

Adverse Childhood Experiences (ACE) study

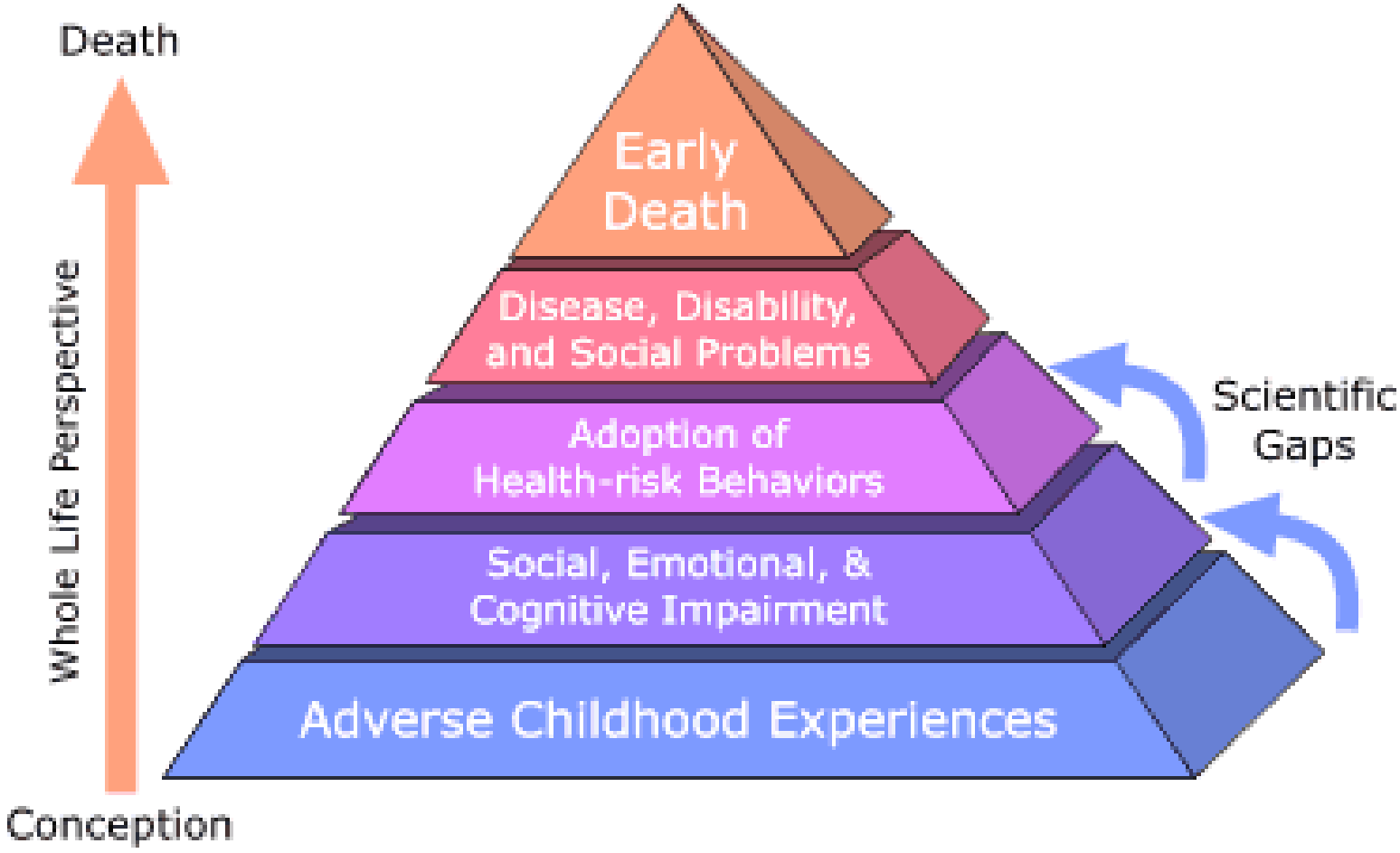
- **Childhood Maltreatment**
  - Abuse & Neglect
- **Household Dysfunction**
  - Spouse abuse
  - Substance abuse
  - Mental illness
  - Separation or divorce
  - Incarceration

- **Health Problems**
  - Alcoholism, substance abuse, smoking
  - Depression, suicide attempts
  - Ischaemic heart disease
  - COPD
  - Liver disease
  - Multiple sexual partners, STDs, unintentional pregnancies

Felitti VJ et al. Am J Prev Med 1998: 14:245-8.

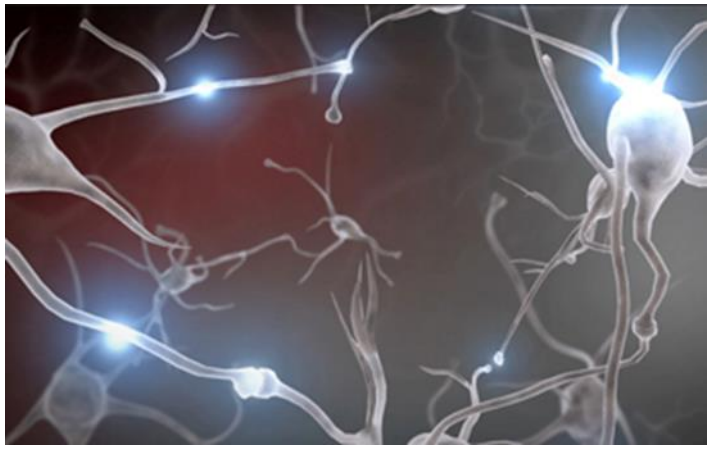
•As the number of ACE increases, the risk for health problems increases.

Adverse childhood experiences

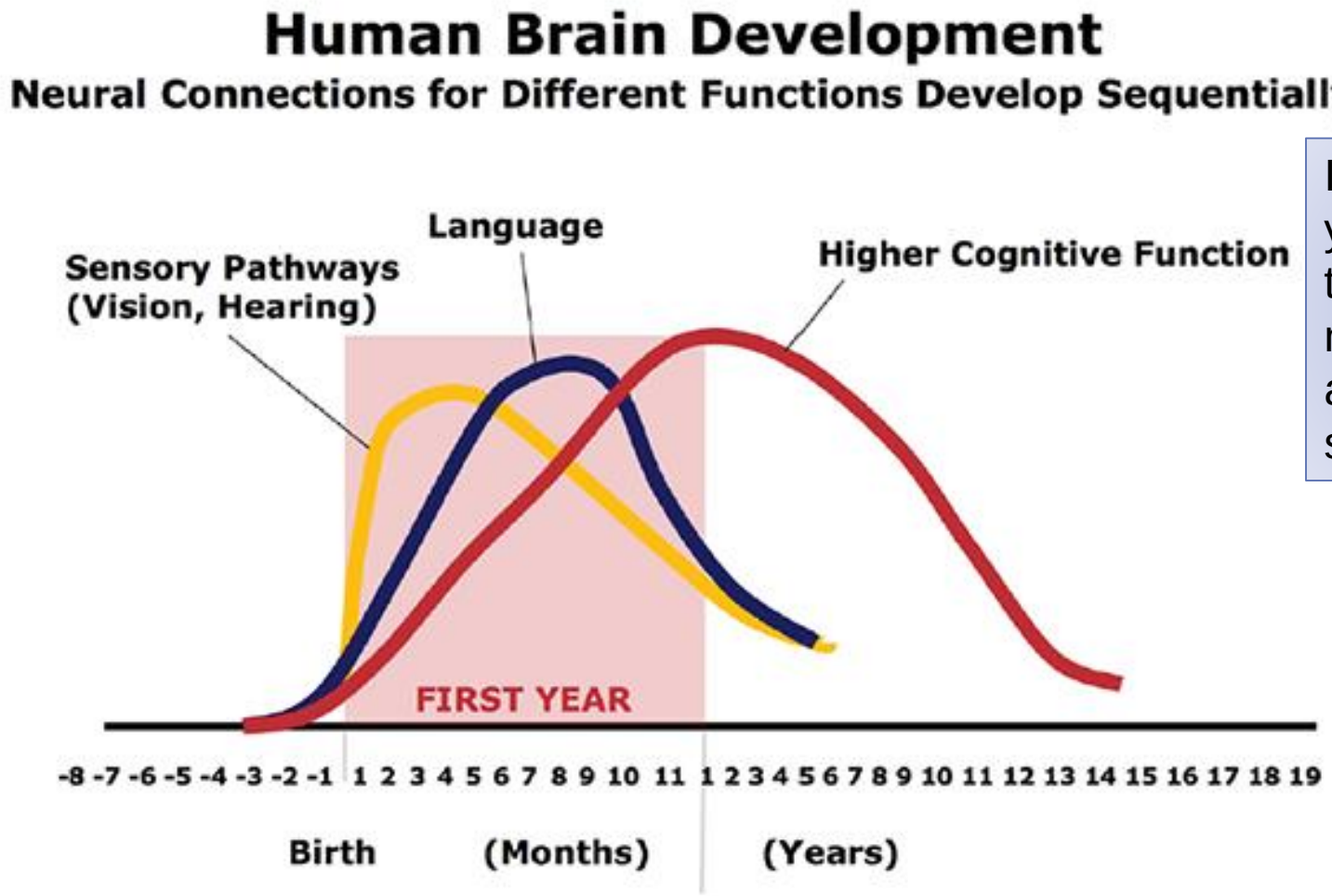


SCIENCE OF EARLY CHILDHOOD DEVELOPMENT

- Early experiences influence the developing brain.
- Plasticity, or the ability for the brain to reorganize and adapt, is greatest in the first years of life and decreases with age.



Center on the Developing Child (2007). The Science of Early Childhood Development (InBrief). Retrieved from www.developingchild.harvard.edu.



In the first few years of life, more than 1 million new neural connections are formed every second.

Pruning
(connections reduced to make more efficient brain circuits)

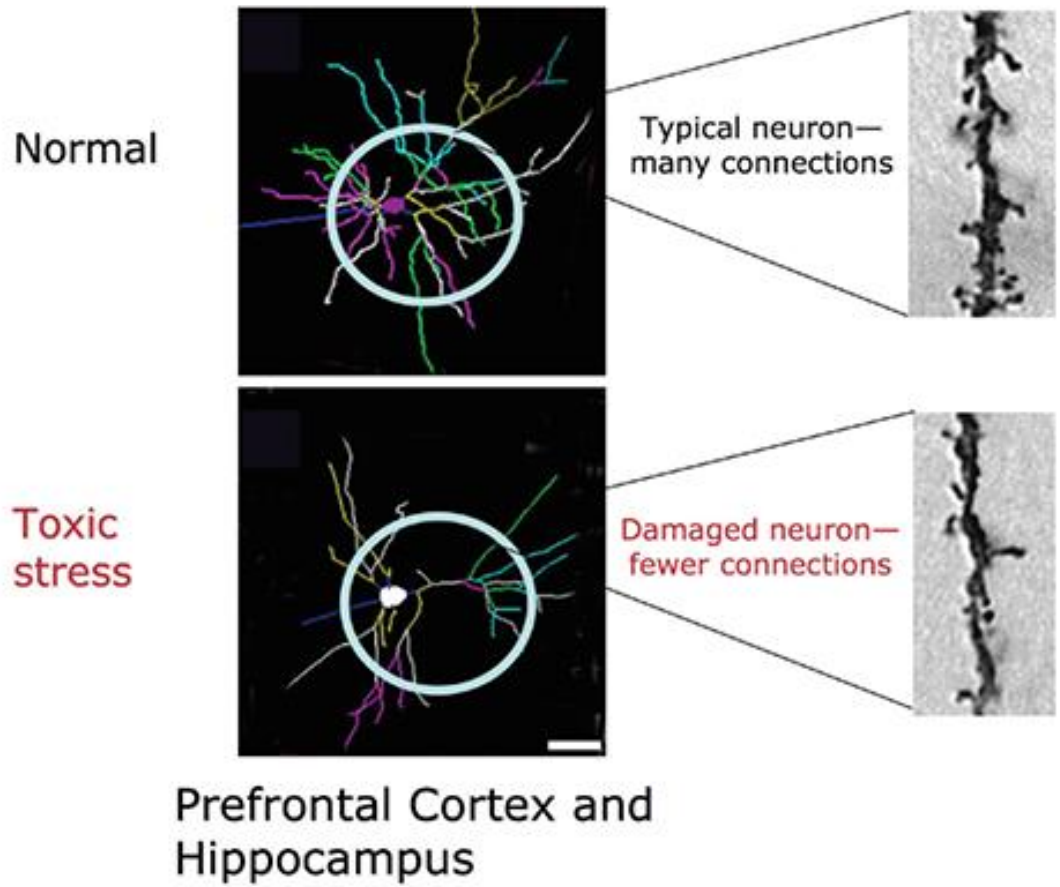
Center on the Developing Child (2007). The Science of Early Childhood Development

The interactive influences of genes and experience shape the developing brain.

Serve and Return Interaction Shapes Brain Circuitry

Center on the Developing Child (2007). The Science of Early Childhood Development

Persistent Stress Changes Brain Architecture



Brains subjected to **toxic stress** have underdeveloped neural connections in areas of the brain most important for successful learning and behavior in school and the workplace.

Center on the Developing Child (2007). The Science of Early Childhood Development

Types of Stress

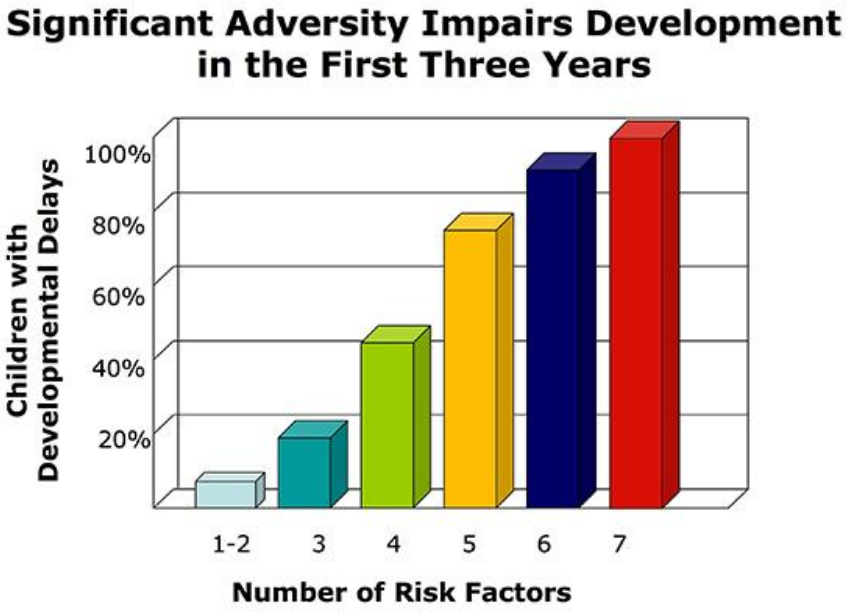
Brief increases in heart rate, mild elevations in stress hormone levels.

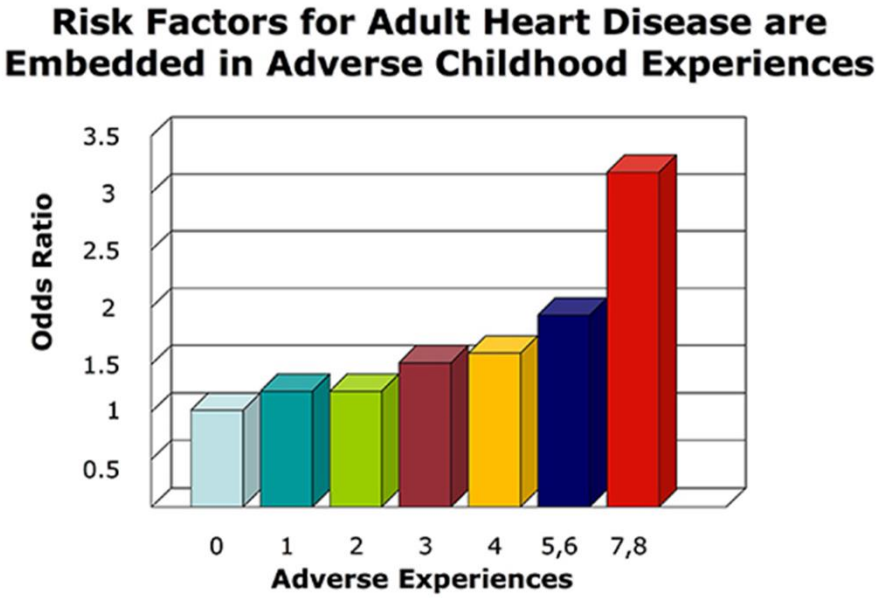
Serious, temporary stress responses, buffered by supportive relationships.

Prolonged activation of stress response systems in the absence of protective relationships.

Toxic stress

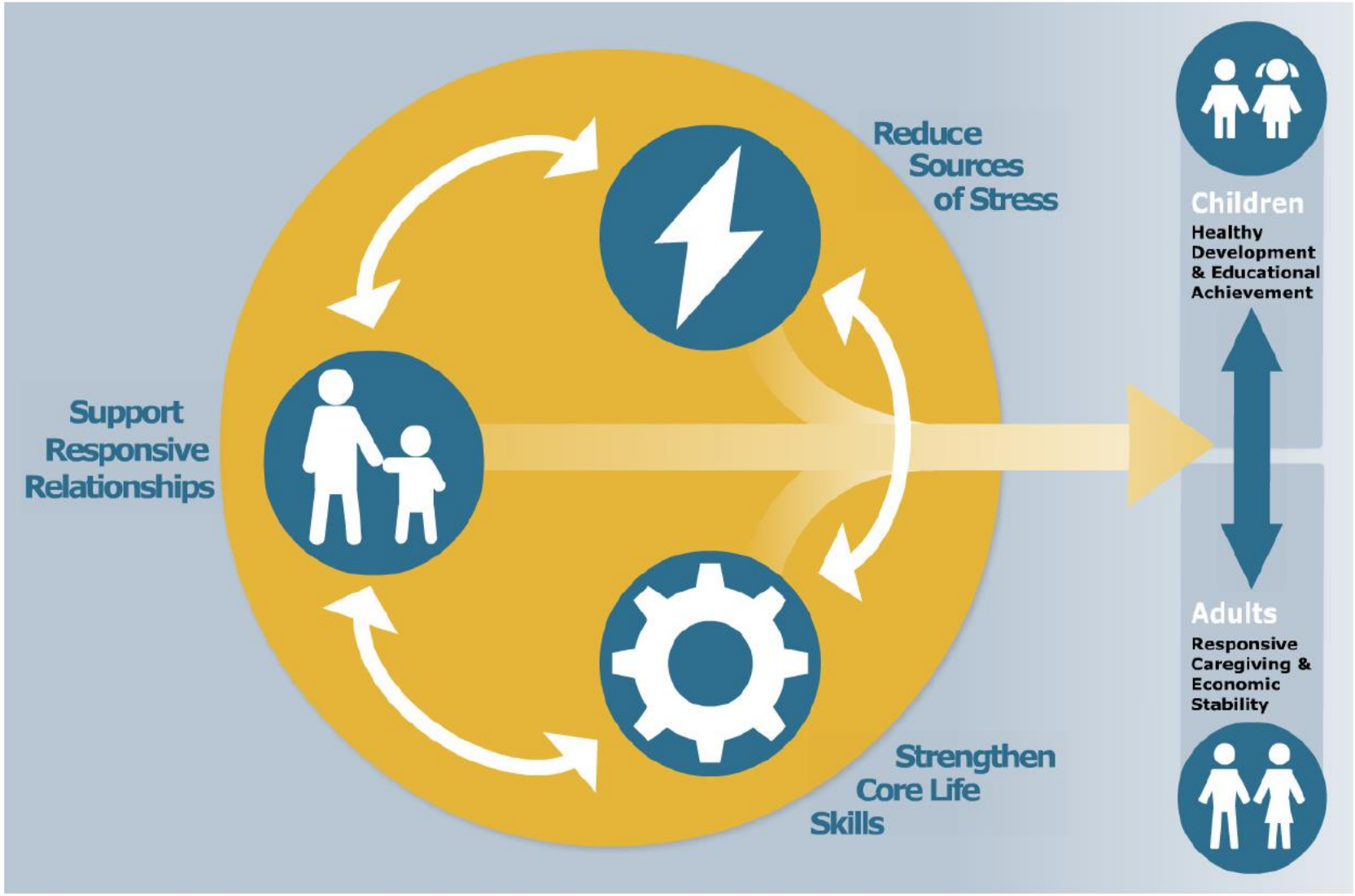
Toxic stress weakens the architecture of the developing brain, which can lead to lifelong problems in learning, behavior, and physical and mental health.





Center on the Developing Child (2007). The Science of Early Childhood Development

Best ways to prevent adverse effect of ACE



Suggested citation: Center on the Developing Child at Harvard University (2017). *Three Principles to Improve Outcomes for Children and Families.* http://www.developingchild.harvard.edu

RECOGNIZING CHILD ABUSE

- If you don’t think about it, you won’t diagnose it.

PHYSICAL ABUSE
身體虐待

Physical Abuse

- a physical injury or physical suffering to a child
- Includes
  - non-accidental use of force,
  - deliberate poisoning,
  - suffocation,
  - burning,
  - Medical child abuse (Munchausen’s Syndrome by Proxy)

Procedures for Handling Child Abuse (Revised 2015), Social Welfare Department, HKSAR

Presentation

- Severe life-threatening injuries
- Delay in seeking medical help in less severe injuries
- Bruises or minor injuries noticed by teachers or nursery staff
- Discovered on routine medical check-up

The Injures

- Superficial injuries/bruises– commonest
- Bone fractures – 2nd
- Burn or scald
- Intracranial injuries
  - Abusive head injuries/ shaken baby syndrome 搖晃嬰兒綜合症
- Injuries to mouth
- Visceral injuries
- Suffocation

ACCIDENTAL OR NON-ACCIDENTAL INJURIES (NAI)?

Inflicted or accidental injuries: HISTORY very important

- Careful history of how the injuries has occurred
- Features suggestive of abuse:
  - Injuries not consistent with history or developmental age of child
  - Unexplained or poorly explained injuries
  - Inconsistent history between care-givers
  - Changing history
  - Delay in seeking help
  - Denial and defensiveness

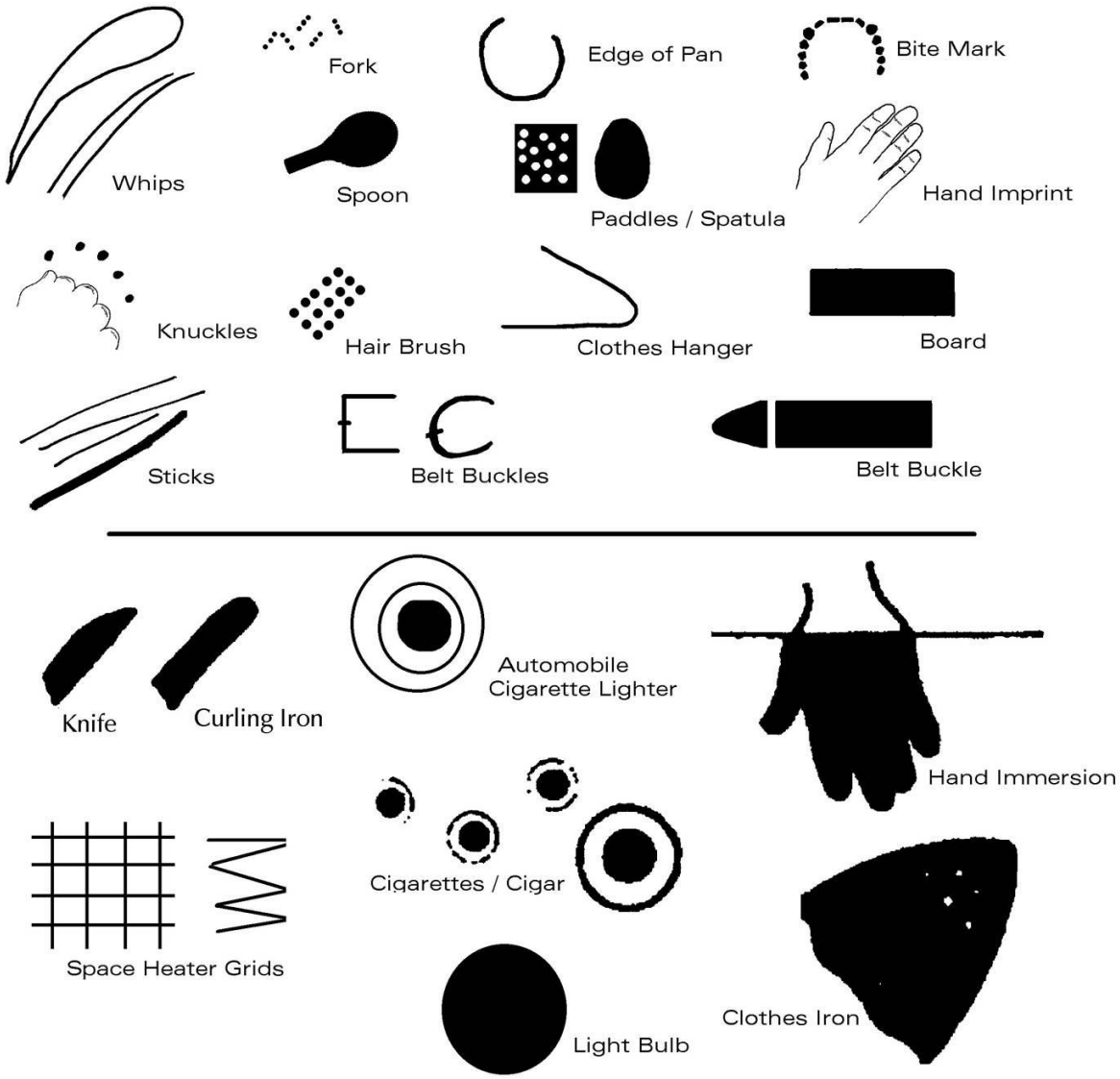
SUPERFICIAL INJURIES

Lee AC. Singapore Med J. 2008 Jun;49(6):445-9

Superficial injuries

- Bruises
  - Suspicious sites: buttocks, abdomen, cheeks, genitalia, legs and arms
  - Absorption of bruises depends on vascularity of affected tissue and extent of injury.
- DO NOT try to age the bruise.
- A yellow bruise is 18-24 hours old.
- Infants who do not pull to standing seldom bruise.

Superficial injuries



Burn injuries

- **cigarette burns**
  - 7-8mm in diameter
- **scalds/immersion injury**
  - *60ºC for 4- 6 seconds causes full-thickness burn*
  - burn of uniform thickness
  - clear demarcation line (glove or stocking)
  - absence of splash marks
  - Doughnut pattern in buttock
  - Sparing of palms, soles, areas between toes, abdominal skin creases

CONDITIONS NOT TO BE MISTAKEN FOR CHILD ABUSE

- Bleeding tendency
  - Idiopathic thrombocytopenic purpura, hemophilia
- Henoch Scholein purpura
- Folk remedies

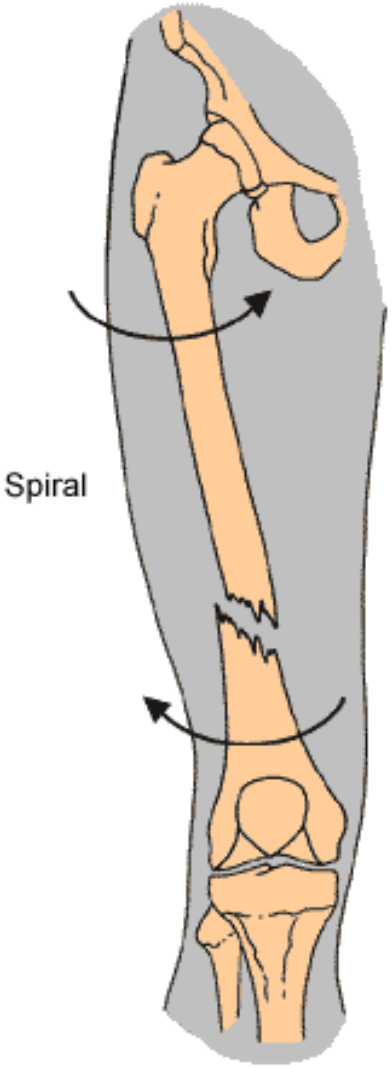
Cupping
拔罐

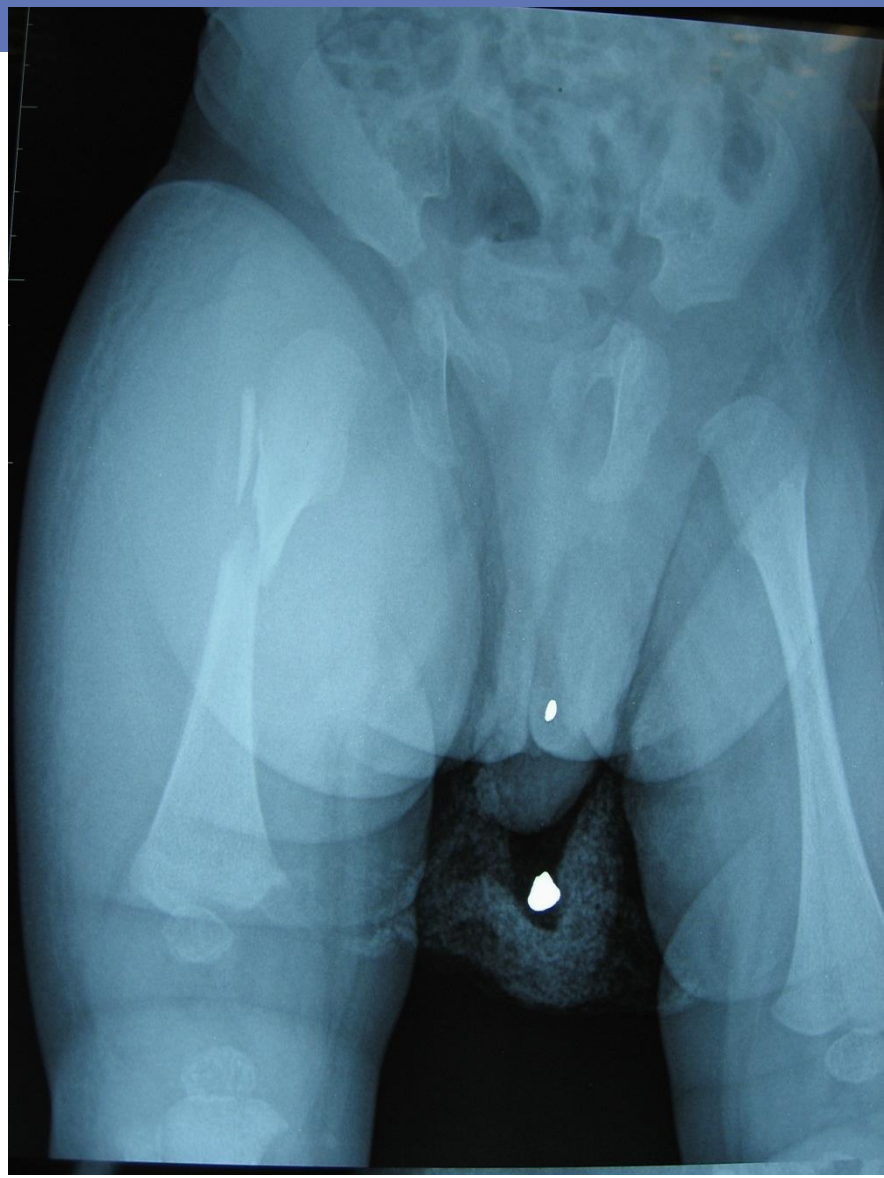
Scraping
刮痧

FRACTURES

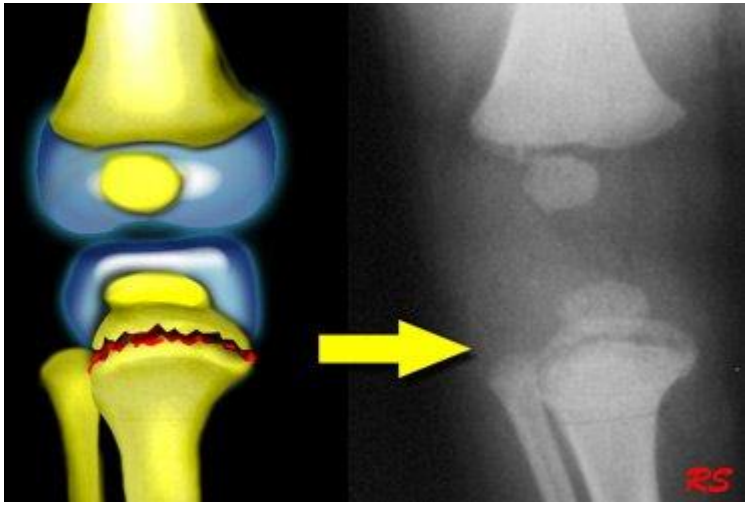
Fractures

- 80% of inflicted fractures are found in infants under 18 months. *(2% of accidental fractures in this age group)*
- 43% of fractures are unsuspected clinically
- Watch out for fracture if
  - Not moving /using the limb
  - Pain
  - Swelling
- Suspect abuse when:
  - > 1 fracture
  - Fractures in multiple sites
  - Fractures in different stages of healing
  - Presence of other injuries
  - History of injury not plausible



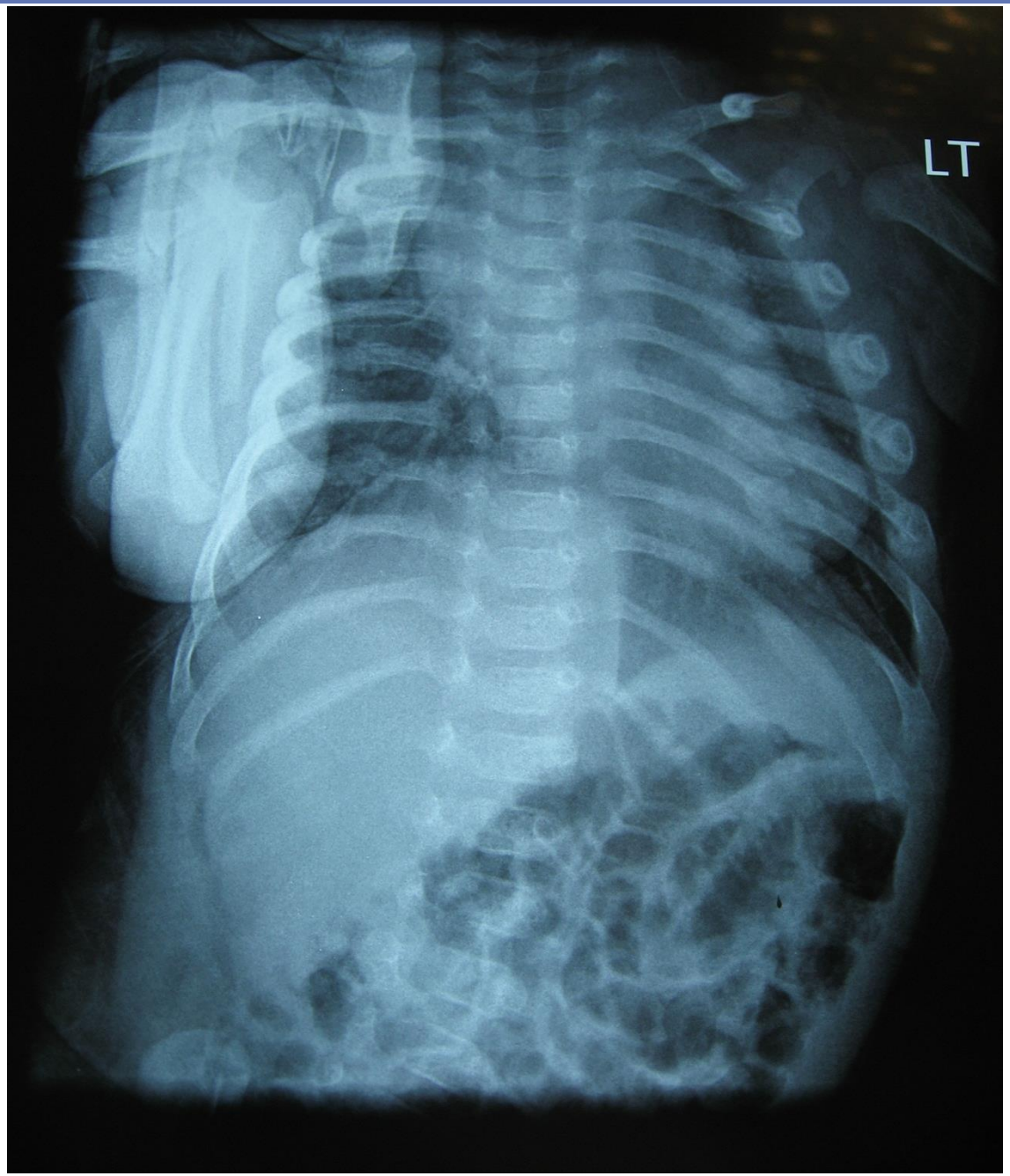


Spiral fractures of the lower extremities in non-ambulatory children are extremely suspicious.



Bucket handle fracture in proximal tibia. The metaphyseal fracture fragment is seen as a disk or bucket handle

Multiple posterior rib fractures



Fractures highly suggestive of abuse

- Classic metaphyseal lesion(CML)
- Posterior rib fracture
- Scapular fracture
- Spinous process fracture
- Sternal fracture

Kleinman PK. Diagnostic imaging of child abuse. Baltimore: Williams & Wilkins, 1998

Dating of fractures

- Fractures without early callus formation: less than 7 - 10 days old
- **Soft callus** visible: after 1st week to 3-4 weeks
- Always perform **skeletal survey** in children < 2 years old in cases of suspected abuse. May repeat after 2 weeks.
- Bone scan helpful in early fractures

INTRACRANIAL INJURY

- Abusive head injury is the most common cause of child abuse deaths
- infants < 12 months old at greatest risks
- injury inflicted by blunt force trauma, shaking or a combintion of forces

任職廚工的年輕父親，聲稱難忍其未足月女兒的喊聲，多次摑打和搖晃女兒，發現女兒反應有異後，即上網搜尋嬰兒不喊的原因，遭女友及女友母親發現後，衝入廚房拿刀並指嚇女友母親。女嬰現時右眼失明，要倚靠呼吸器呼吸，如同植物人。年輕父親承認襲擊兒童和刑事恐嚇等罪。被告今（4日）被判刑6年零4個半月。

2018-09-04

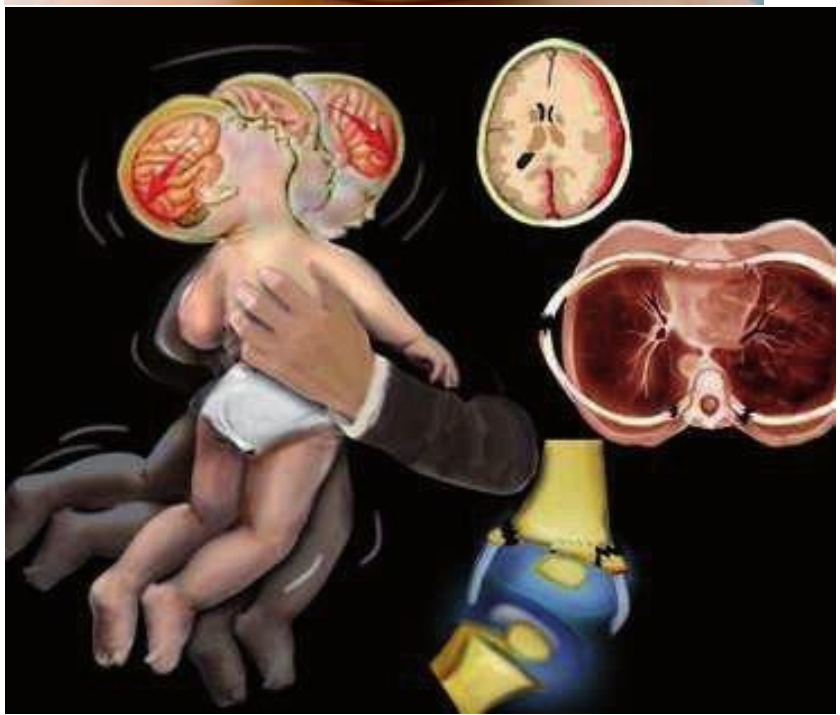
Abusive Head Injury (AHI)

- “Shaken baby syndrome” 搖晃嬰兒綜合症 - a constellation of clinical findings in infants:
  - retinal hemorrhages
  - Subdural hemorrhages &/or subarachnoid hemorrhages
  - Cerebral edema
  - little evidence of external cranial trauma
- Brain injury - due to sudden angular deceleration
- Perpetrators who have confessed
  - Report multiple episodes of shaking over weeks to months
  - 91% of children developed symptoms immediately after injury

Abusive Head Injury (AHI)

- Trigger for the shaking episodes: crying

- Infants < 6 months old especially vulnerable
  - head large in proportion to body size
  - weak neck muscles
  - fragile, undeveloped brains

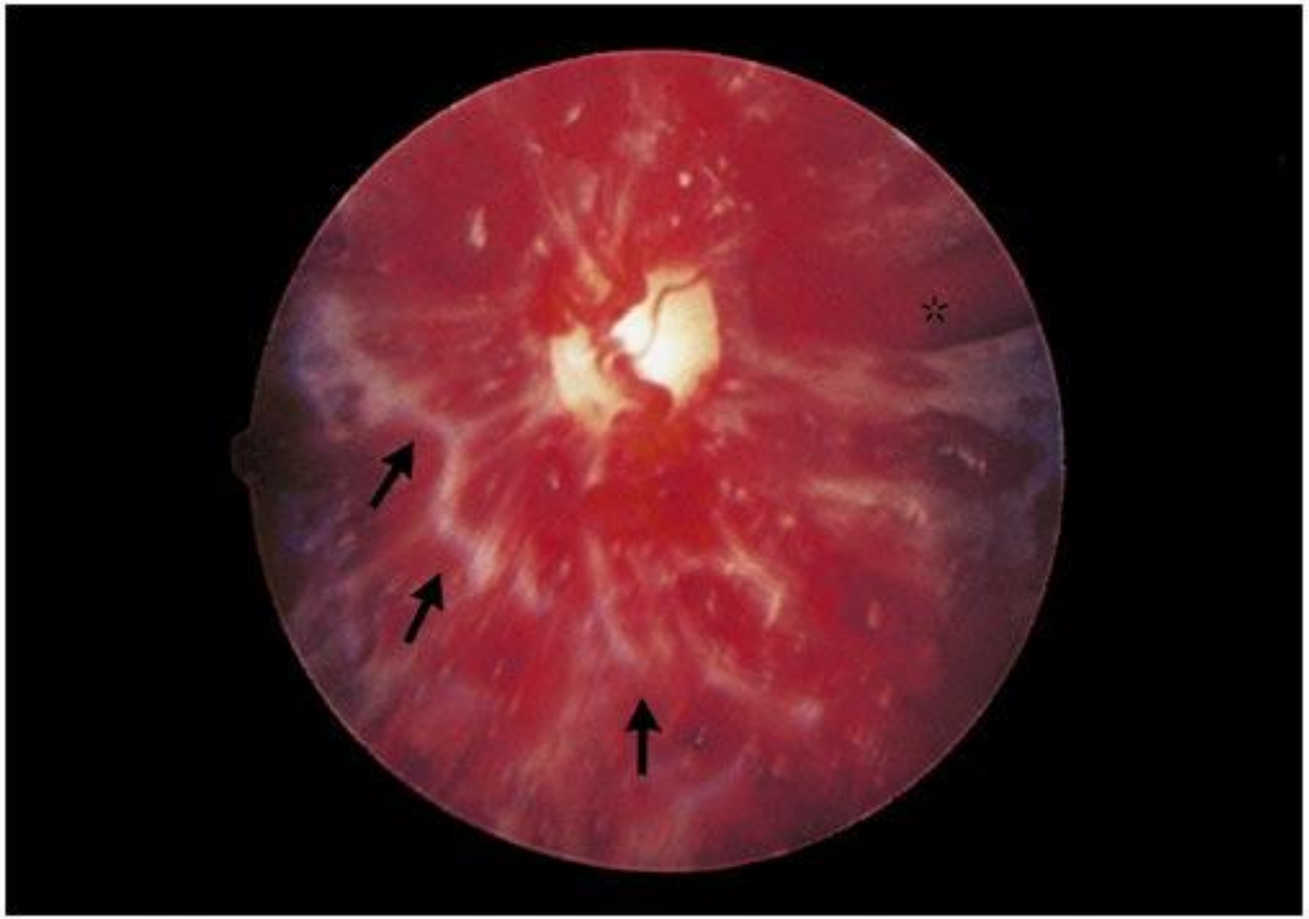


Clinical features of abusive head injuries

- history often unreliable
- in <3 years old (usually in 1st year)
- common symptoms: lethargy, irritability, impaired consciousness, vomiting, poor feeding, breathing difficulties and apnea
- Seizures (40-70%)
- Retinal hemorrhages (65-95%)
- Bruises +/- : grip marks over upper arms/chest

A five-month-old girl was admitted to the hospital because of uncontrollable seizures

Circum-papillary retinal ridge

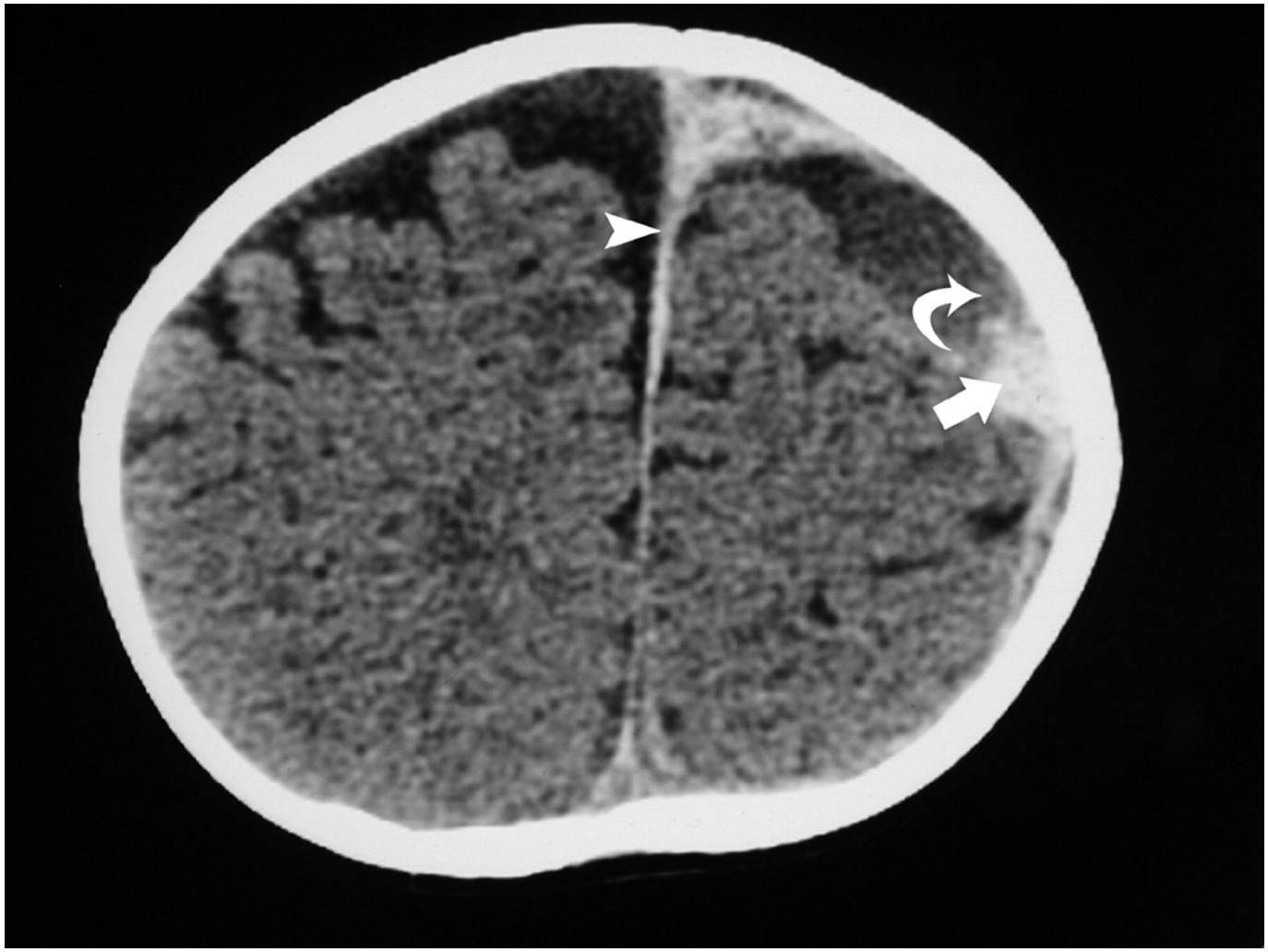


Multiple intraretinal & preretinal hemorrhages

Hylton, C. et al. N Engl J Med 2004;351:170

The NEW ENGLAND JOURNAL of MEDICINE

Noncontrast CT scan performed on 13-month-old child who had sustained NAHI shows heterogeneous left frontal convexity and anterior interhemispheric (arrowhead) SDH



Tung, G. A. et al. Pediatrics 2006;118:626-633

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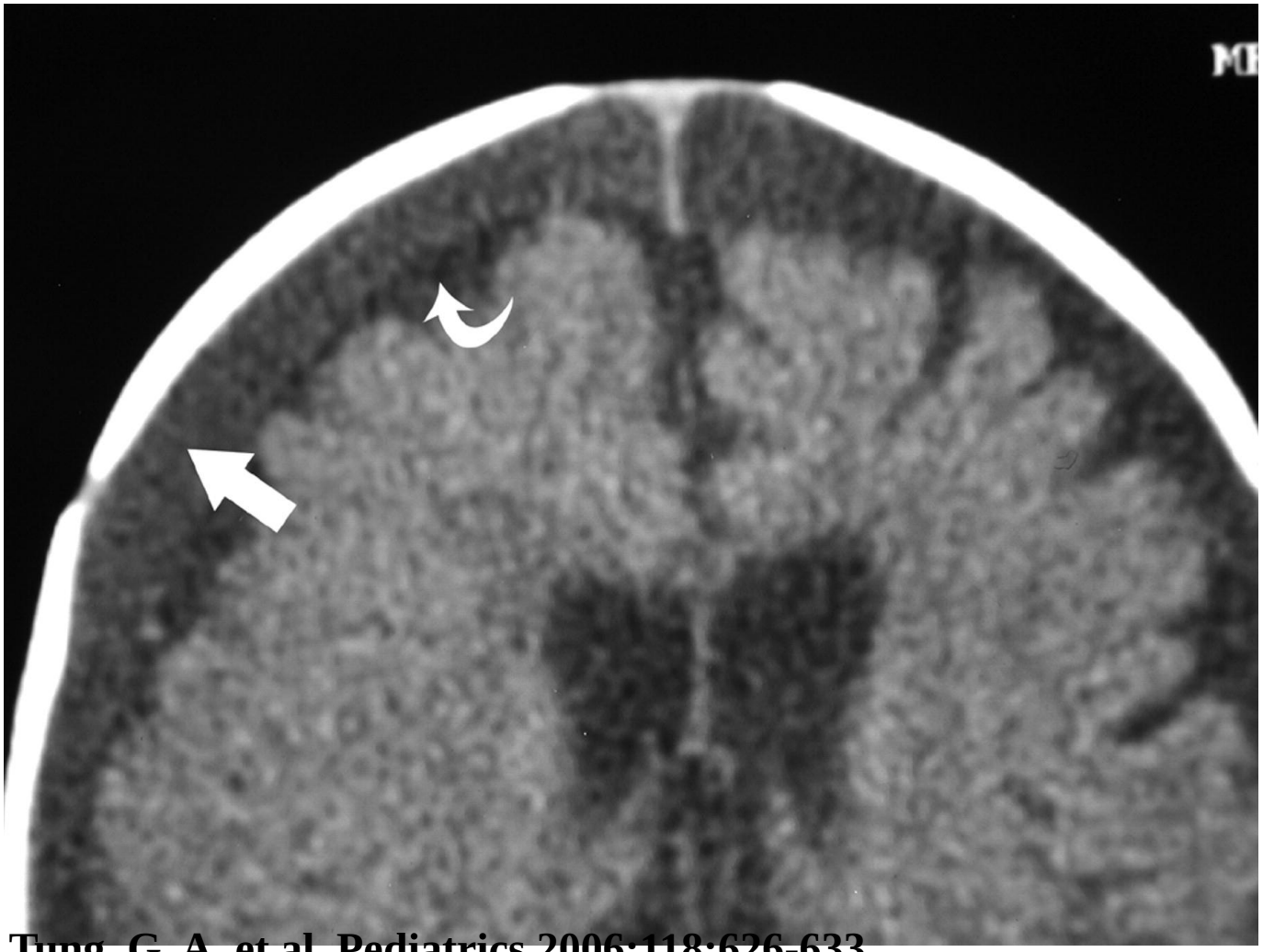
Noncontrast CT scan on 6-month-old child after NAHI shows bilateral homogeneous SDHs (arrow) that are lower in radiodensity compared with adjacent cerebral cortex but higher in density than CSF in subarachnoid space (curved arrow) and lateral ventricles



Tung, G. A. et al. Pediatrics 2006;118:626-633

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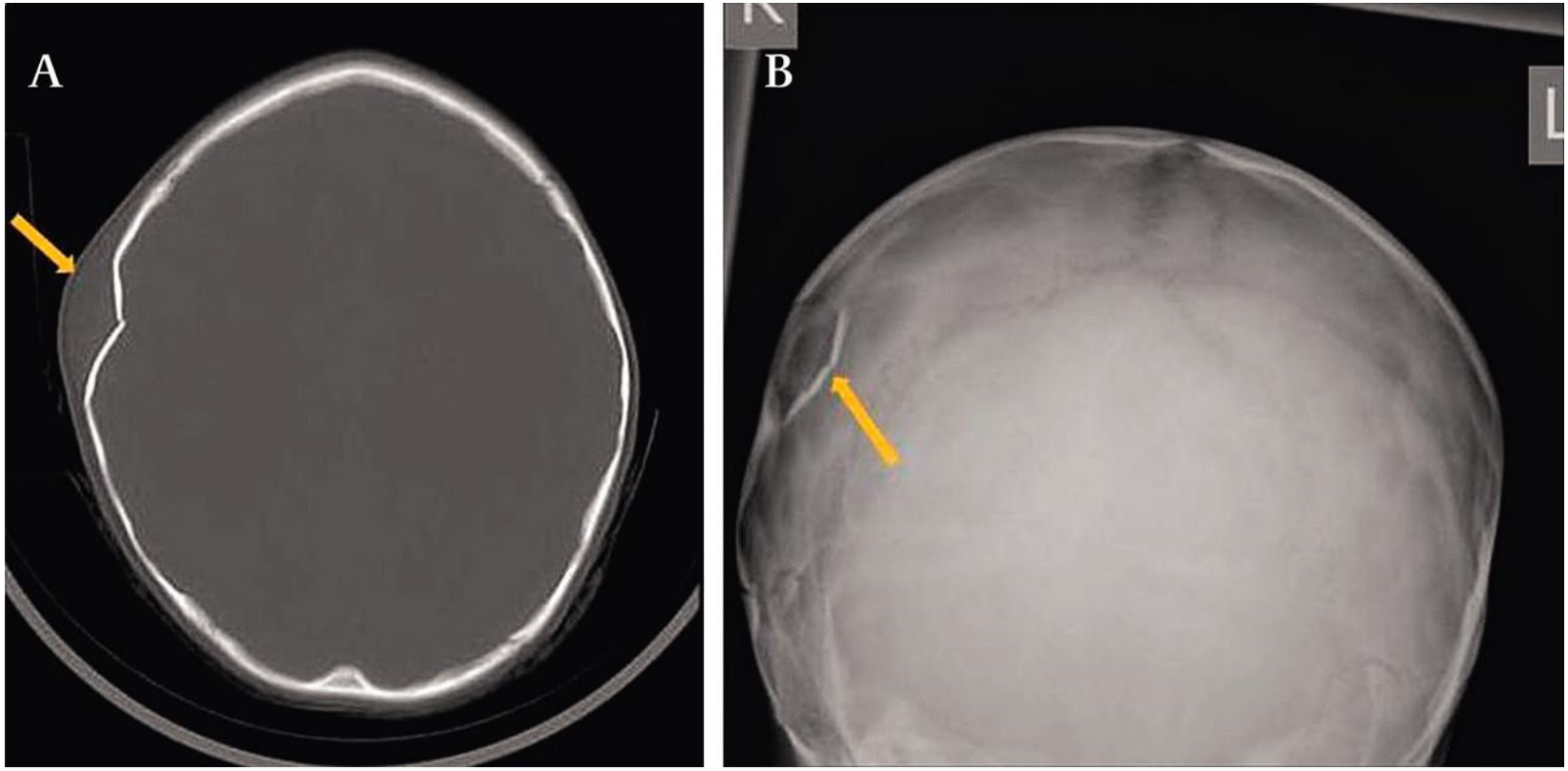
Skull fractures

- Accidental skull fractures from short falls < 4 ft
  - Single linear fractures
  - Parietal bone most commonly involved
- *Features suspicious of abusive skull fractures*
  - Multiple or complex fractures
  - Depressed fractures
  - Diastatic fractures (max # width >3mm)
  - Growing fracture
  - Involving more than one cranial bone
  - Non-parietal fracture
  - Associated intracranial injury

CT head scan (A) and plain skull radiograph (B) of a child with a depressed skull fracture.



Rajaram S et al. Postgrad Med J 2011;87:355-361

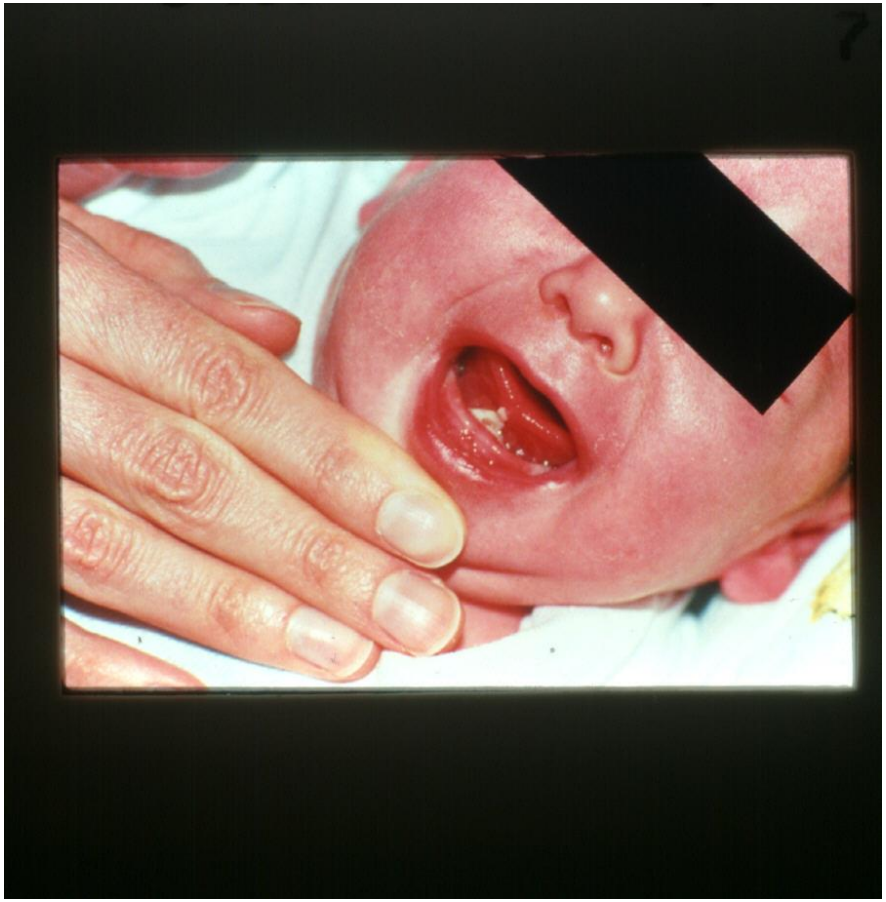
PMJ

Outcome of abusive head injuries

- Mortality: 25%
- UP to 80% of survivors suffer from lifelong disabilities
  - Small head and brain atrophy (61-100%)
  - Visual impairment (18-48%)
  - Intractable epilepsy (11-32%)
- Those that were comatose on presentation, 60% died or had profound mental retardation, cerebral palsy

Injuries to the mouth

- Broken teeth in older children
- Torn frenulum in infants
- Pharyngeal injuries

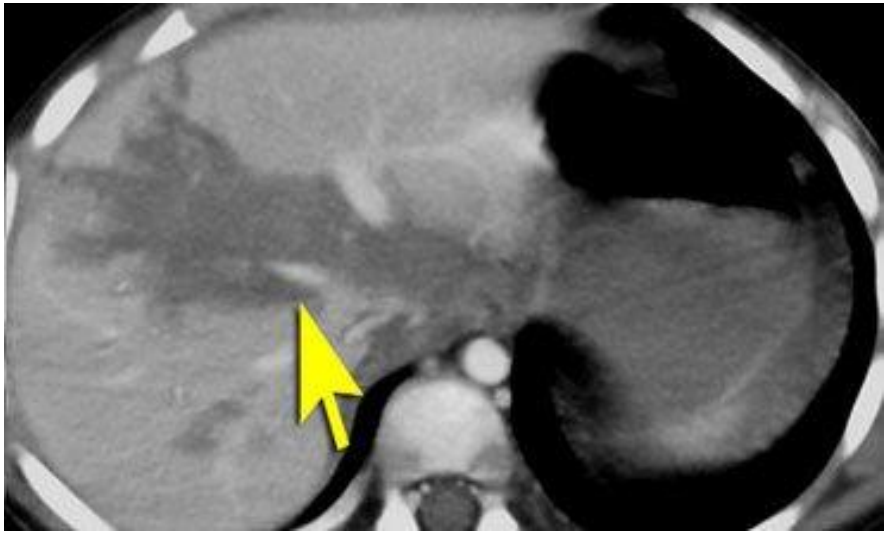


Abdominal Injuries

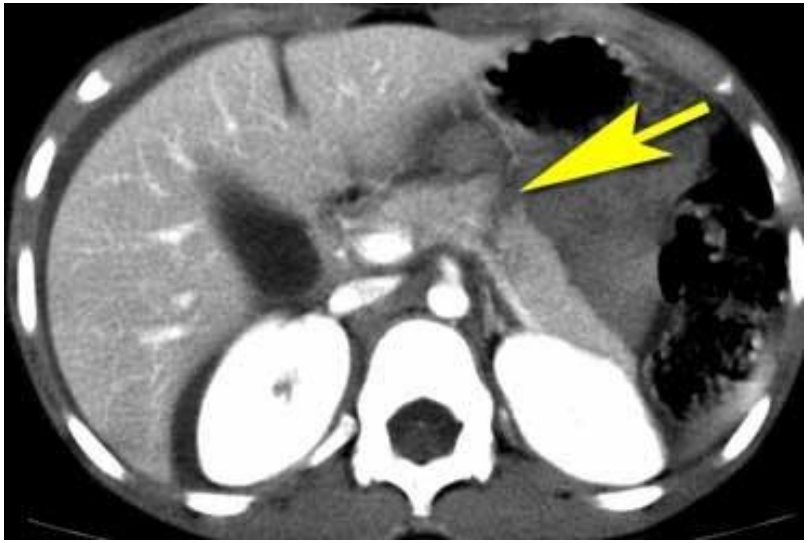
- 2nd most common cause of death
- Reported case fatalities 40-50%
- Mechanism of injury:
  - direct blows (crushing of organs, hollow viscus perforation)
  - indirect shearing forces
- High morbidity: delay in presentation & diagnosis
- Multiple visceral injuries common

Abdominal injuries

- Perforation of small intestine (* jejunum)
- Tear of mesentery
- Solid organs: liver > kidney
- Splenic injuries rare
- Pancreatic injuries not uncommon



Liver laceration



Pancreatic laceration

www.radiologyassistant.nl

Investigations for physical abuse

- Blood tests
  - Full blood count
  - coagulation screen
  - liver and renal function tests, amylase
  - Urine microscopy for RBC
- Radiology
  - **Full skeletal survey** (SXR, CXR, XR 4 limbs, XR spine**) in < 2 years old**: repeat imaging at 10-14 days; bone scan
  - CT scan
  - MRI scan

MEDICAL CHILD ABUSE/ MUNCHAUSEN SYNDROME BY PROXY 照顧者假裝兒童生病求醫

- Fabricated or induced illness

Munchausen Syndrome by Proxy

- Characteristics:
  - 1) a parent or other caregiver fabricates an illness;
  - 2) the child is presented persistently for medical assessment, often resulting in multiple procedures;
  - 3) the perpetrator denies the cause of the child's illness; and
  - 4) acute symptoms and signs of the illness stop when the child and perpetrator are separated
- seen more commonly in younger children

Munchausen Syndrome by Proxy

- mothers - the sole perpetrators in 94% to 99% of case
  - Many of the abusing mothers have a history of psychiatric illness
  - have friendly demeanors, some degree of training in the medical field, and appear very involved in the care of their children during hospitalization

Medical child abuse/ Munchausen Syndrome by Proxy

- **Warning signs**
  - Illness unexplained, prolonged
  - Discrepancy between symptoms/signs and history
  - Inappropriate or incongruous symptoms or signs or appears only when mother is attending
  - Children alleged to be allergic to a no. of drugs or food
- **Diagnosis**
  - High index of clinical suspicion
  - Objective verification of medical history
  - Review of all medical records
  - Monitor social media
  - Video surveillance

CHILD SEXUAL ABUSE

Child sexual abuse

- Involvement of children in sexual activities that…
  - They cannot understand
  - They are not developmentally prepared for
  - They cannot give informed consent for
  - Violate societal taboos
- Perpetrators
  - Usually known to child
  - Intend to maintain secrecy

Definition of child sexual abuse

- Involvement of a child in sexual activity (e.g. rape, oral sex) which is **unlawful**, or to which a **child is unable to give informed consent**.

- **Child sexual abuse differentiates from casual sexual relationship that does not include any sexual exploitation** e.g. between a boy and a girl, though the boy can be liable for offences like indecent assault or unlawful sexual intercourse with an underaged girl.

Procedural Guide For Handling Child Abuse Cases (Revised 2015), SWD HK

- 'Involves **forcing or enticing** a child or young person to take part in sexual activities, not necessarily involving a high level of violence, whether or not the child is aware of what is happening.

  - The activities may involve **physical contact**, including assault by *penetration* (for example, rape or oral sex) or *non-penetrative* acts such as masturbation, kissing, rubbing and touching outside of clothing.

  - They may also include **non-contact activities**, such as involving children in looking at, or in the production of, sexual images, watching sexual activities, encouraging children to behave in sexually inappropriate ways, or grooming a child in preparation for abuse (including via the internet). Sexual abuse is not solely perpetrated by adult males. Women can also commit acts of sexual abuse, as can other children.'

Working together to safeguard children 2015, Department of Education, UK

Presentation to medical care

- Disclosure of inappropriate sexual contact
- Behavioral concerns
- Physical injury to genitals
- Genitourinary symptoms
- Sexually transmitted infections (STIs)

Suspicion of child sexual abuse

- **Direct disclosure by the child**
- **High suspicion**:
  - Genital injuries not consistent with accident.
  - Severe psychiatric disturbance, such as mutism, eating disorder, suicide, self-mutilation.
  - Repeated and frequent sexualized behaviour.
  - Marked frozen behaviour.
  - Worrying information from adults

Suspicion of child sexual abuse

- **Medium** :
  - Combination of some recurrent medical symptoms
    - e.g. unexplained vaginal soreness or bleeding, urinary tract infections, sleep disturbances, anorexia
- **Low** :
  - Isolated observation of sexualized behaviour.
  - Single physical symptom <u>without</u> manifestation of behavioural or emotional problems.
    - E.g. recurrent urinary tract infection, vaginal/penile discharge

•Ref.: Camden Procedures in cases of suspected child sexual abuse

Acting on suspicion

Any concern regarding risk of harm to a child will always override a professional duty of confidentiality

Initial thought:
"Could this be abuse ?"

Assessment:
- History
- Physical examination
-Investigations

Making a report:
"reasonable cause to suspect"

Child Protection

Address 5 important issues

- **The child's safety**
  - safe to go home? will child be punished by disclosing abuse?
- **Reporting to child protection authorities**
  - informing social worker +/- police
- **The child's mental health**
  - symptoms of depression and post traumatic stress disorder
  - trauma symptoms in children were highly associated with the degree of self-blame the child felt about the abuse incident(s)
- **The need for a physical examination**
  - to rule out injury
  - need to be done by doctor who is experienced for the genital examination
- **The need for forensic evidence collection**
  - children who have had recent sexual contact **involving exchange of body fluids** should be immediately referred to forensic pathologist capable of collecting **evidence**, **best to be collected within 72 hours**

Medical examination in sexual abuse

- Consent
- Prepare the child. Avoid repetitive genital examination.
- Colposcope
- Photographic documentation

Conclusive or very strong evidence of sexual abuse

- Semen, blood or hair foreign to the child within the vagina or anus
- Sexually transmitted diseases - e.g. gonorrhoea, syphilis, HIV, Chlamydia trachomatis

STD Confirmed	Sexual Abuse	Suggested Action
Gonorrhea*	Diagnostic**	Report#
Syphilis*	Diagnostic	Report
HIV infection##	Diagnostic	Report
C trachomatis infection*	Diagnostic**	Report
T vaginalis infection	Highly suspicious	Report
C acuminata infection* (anogenital warts)	Suspicious	Report
Herpes simplex (genital location)	Suspicious	Report^
Bacterial vaginosis	Inconclusive	Medical follow-up

* If not perinatally acquired and rare nonsexual vertical transmission is excluded

** Although the culture technique is the "gold standard," current studies are investigating the use of nucleic acid-amplification tests as an alternative diagnostic method in children.

To the agency mandated in the community to receive reports of suspected sexual abuse

If not acquired perinatally or by transfusion

^Unless there is a clear history of autoinoculation

CDC

Genital Examination of the Sexually Abused Child

- NORMAL exams are the NORM
  - For both girls and boys
  - Abuse may not have injured the genitals.
  - Abuse may not have involved the genitals.
  - Injuries may have healed.
- Positive clinical findings were more likely when children were examined within 72 hours after the abuse

Berenson A.et al: A case-control study of anatomic changes resulting from sexual abuse. Am J Obstet Gynecol 2000; 182: 820-834
Adams J.A et al: Examination findings in legally confirmed child sexual abuse: it's normal to be normal. Pediatrics 1994; 94: pp. 310

NEGLECT

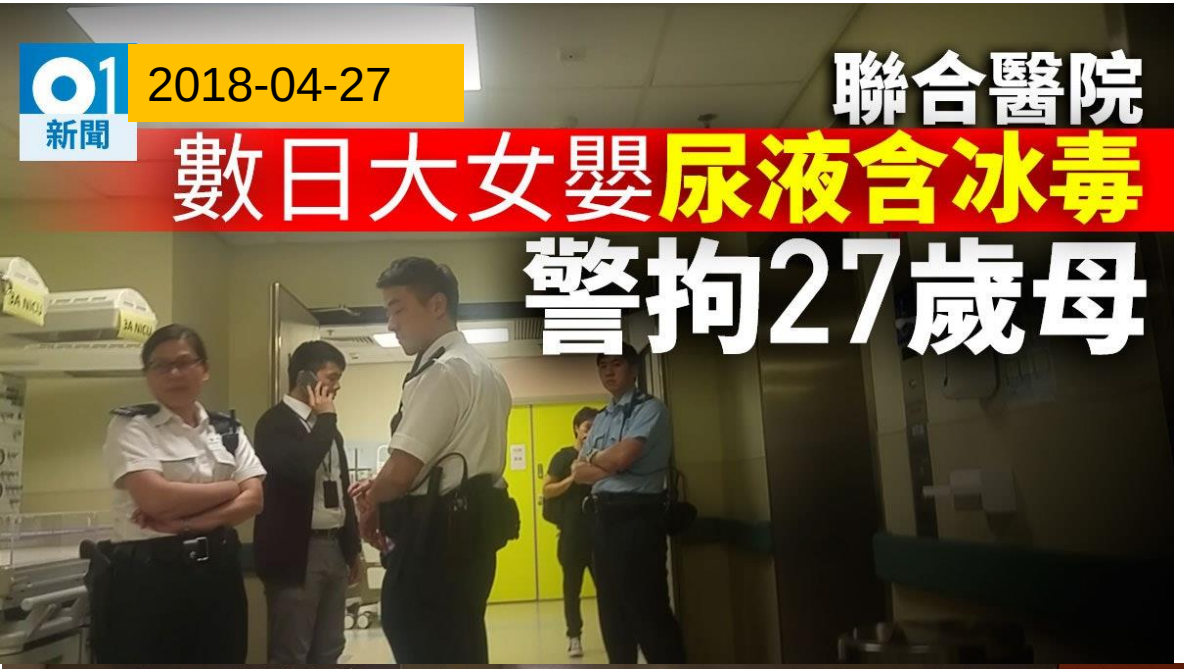
A 3 month-old girl starved to death

法官：港嬰餓死難以置信

Mingpao 28-5-2010

- 一對領取綜援的同居男女餓死3個月大女嬰，被判監6年。法官判刑時**說**，香港有嬰兒因飢餓致死，難以置信。
  - 他批評男被告聲稱無錢買奶粉，但自己就有錢買煙，又以沒有為女兒申請出世紙作藉口，而無申請更多綜援金，反映兩人懶惰，導致嬰兒失去生命…
- 女嬰出生時2.89公斤，健康正常。
  - 去年6月26日，黃報警指3個月大的女兒在天水圍的寓所中昏迷，送院時女嬰已沒有呼吸脈搏，送院後證實死亡。法醫在驗屍時更發現她體**內幾近沒有**脂肪。
- 女嬰當時體重僅2.68公斤，比出生時還要輕磅….

- Birth weight: 2.89kg
- 3 months old: 2.68kg
- Unmarried parents on CSSA
- Use diluted milk to feed the baby
- Baby was found to be comatose at home


01 新聞
2018-04-27
聯合醫院
數日大女嬰尿液含冰毒
警拘27歲母

N8 北翼八樓
2018-08-15
01 新聞
兒科(內科)病房
兒科(外科)病房
紅十字會學校
Red Cross Hospital School
21月大男嬰尿液含毒品
26歲父涉虐兒被捕 警追緝母親

17日大女嬰尿液含冰毒　兩孩之母認虐兒判感化

- 染有毒癮的36歲婦人，**在她剛誕下的女嬰身旁吸冰**，女嬰僅17日大，已驗出尿液含冰毒。女嬰母早前在西九龍法院承認一項虐兒罪，母親堅稱已改過，多次驗尿亦證未見吸毒，裁判官今（23日）判刑時，向女嬰母明言她現時已是兩名孩子之母，吸毒對孩子及她自己都會造成很大的傷害，勸籲她不要再吸毒，並判處她12個月感化令。

- 2016年 6 月**於朋友家中誕下女嬰**，醫院社工其後上門將當時僅17日大的女嬰送往醫院檢查，發現女嬰尿液內含冰毒成分，被告曾在女嬰4呎範圍內吸食冰毒。女被告**去年底亦誕下另一名孩子**，該嬰兒則未受毒品影響。

23-2-2018 HK01

Neglect

- is severe or a repeated pattern of lacking of attention to a child's basic needs that endangers or impairs the child's health or development.
  - *Severity, chronicity, frequency, intentionality*
  - *Cultural context* e.g. young children as carers for sibs
- Neglect may be :
  - Physical
    - e.g. failure to provide necessary food, clothing or shelter, failure to prevent physical injury or suffering, lack of appropriate supervision or left unattended
  - Medical
  - Educational
  - Emotional

Severe neglect

- Chronic neglect can alter the development of biological stress response systems in a way that compromises children's ability to cope with adversity.
- Children who have experienced severe neglect are more likely to have cognitive problems, academic delays, deficits in executive function skills, and difficulties with attention regulation.
- The negative consequences of severe neglect can be reduced or reversed through appropriate and timely interventions.

National Scientific Council on the Developing Child. (2012). The Science of Neglect: The Persistent Absence of Responsive Care Disrupts the Developing Brain: Working Paper 12. http://www.developingchild.harvard.edu

PSYCHOLOGICAL ABUSE

Psychological abuse

- acts that damage immediately or ultimately child's behavioural, cognitive, affective, or physical functioning
- include caregiver acts of
  - spurning (Hostile Rejecting / Degrading)
    - verbal and non-verbal acts
  - terrorizing
    - behaviour that threatens or is likely to physically hurt, kill, abandon, or place the child or child's loved ones or objects in recognizably dangerous situations.
  - Isolating
  - Exploiting/corrupting
    - acts that encourage the child to develop inappropriate behaviour (self-destructive, anti-social, criminal, deviant, or other maladaptive behaviour).
  - Denying emotional responsiveness (ignoring)

MULTIPLE ABUSE

【短暫生命】5歲臨臨小小身軀遺30多個傷口　父母控謀殺明提堂

五歲女童臨臨疑在家被親生父親以及繼母虐打致死，其兄長亦被發現傷痕累累 6 Jan 2018

Abused girl, 5, hit ceiling after being hurled in air and was poked with scissors, Hong Kong court told in murder case

- Prosecutors reveal horrific abuse inflicted on victim and older brother by father and stepmother:
  - "The couple held the girl under her arms and **tossed her up a dozen times, causing her head to hit the ceiling**. They also **swung her by holding both her arms**,"
  - …were assaulted almost daily. Their arms and legs were **beaten with canes**. **Their faces were smacked with slippers**. **Their chests were poked with scissors**…
  - **often not given enough food**
  - often asked to **sleep on the floor of the living room without thick blankets**
- the deceased girl and her elder brother – who was also found **covered with bruises and scratches**

SCMP 09 January, 2018

MANAGEMENT OF CHILD ABUSE

- **Multidisciplinary** approach
- Safety and welfare of the child
- Good pediatric care:
  - Treat injuries and obtain help for psychological problems.
  - Assist in protecting the child from further abuse.

Multi-disciplinary approach

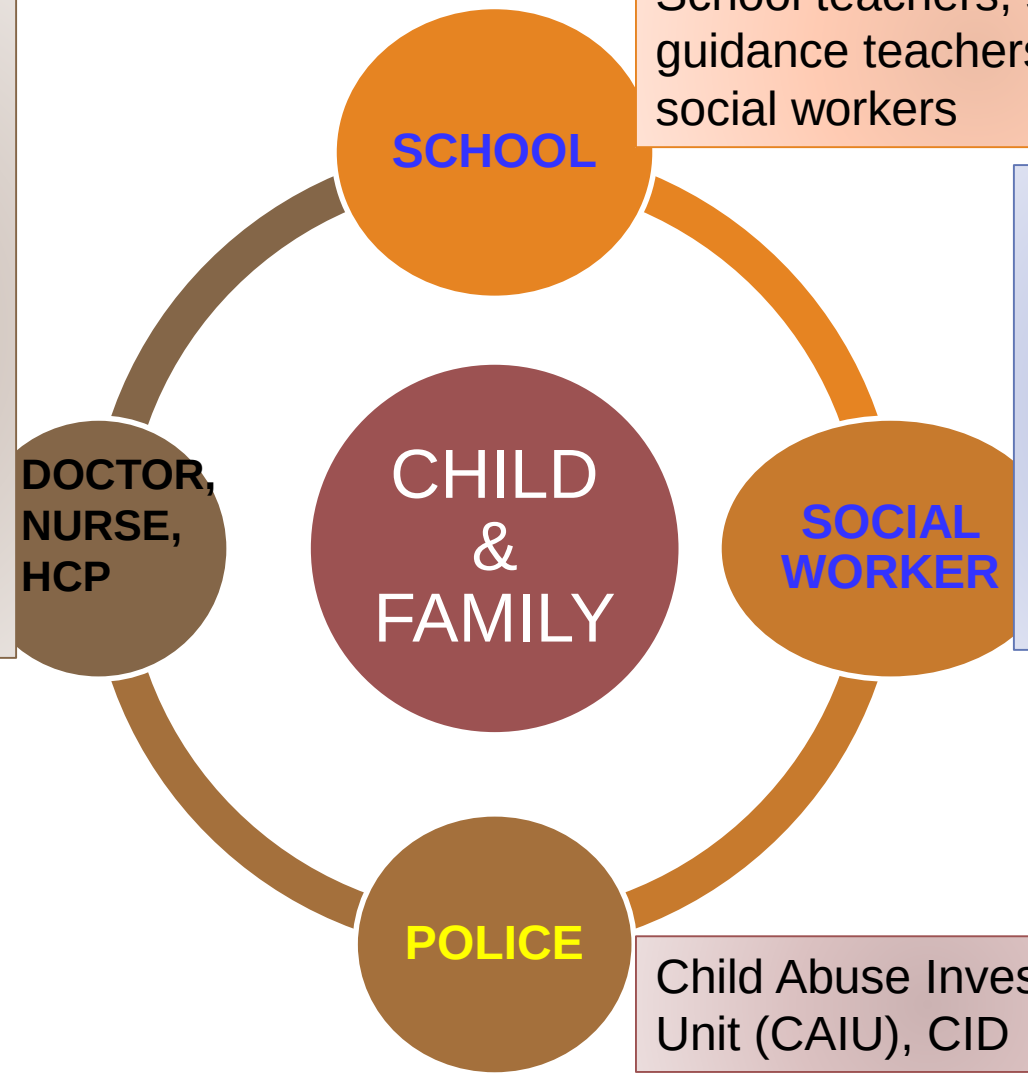
1. Case identification and initial assessment
2. Crises intervention and investigations
3. Welfare plan and support services

Who are involved?

Emergency
Medicine
Paediatrics
Orthopaedics
Psychiatry
Surgery
Neurosurgery
Ophthalmology
O&G
Clinical Psychology
Forensic Pathology

School teachers, school guidance teachers, school social workers

SWD: Family Child Protective Services Unit (FCPSU), Integrated Family Services Centre (IFSC), NGO Medical Social workers



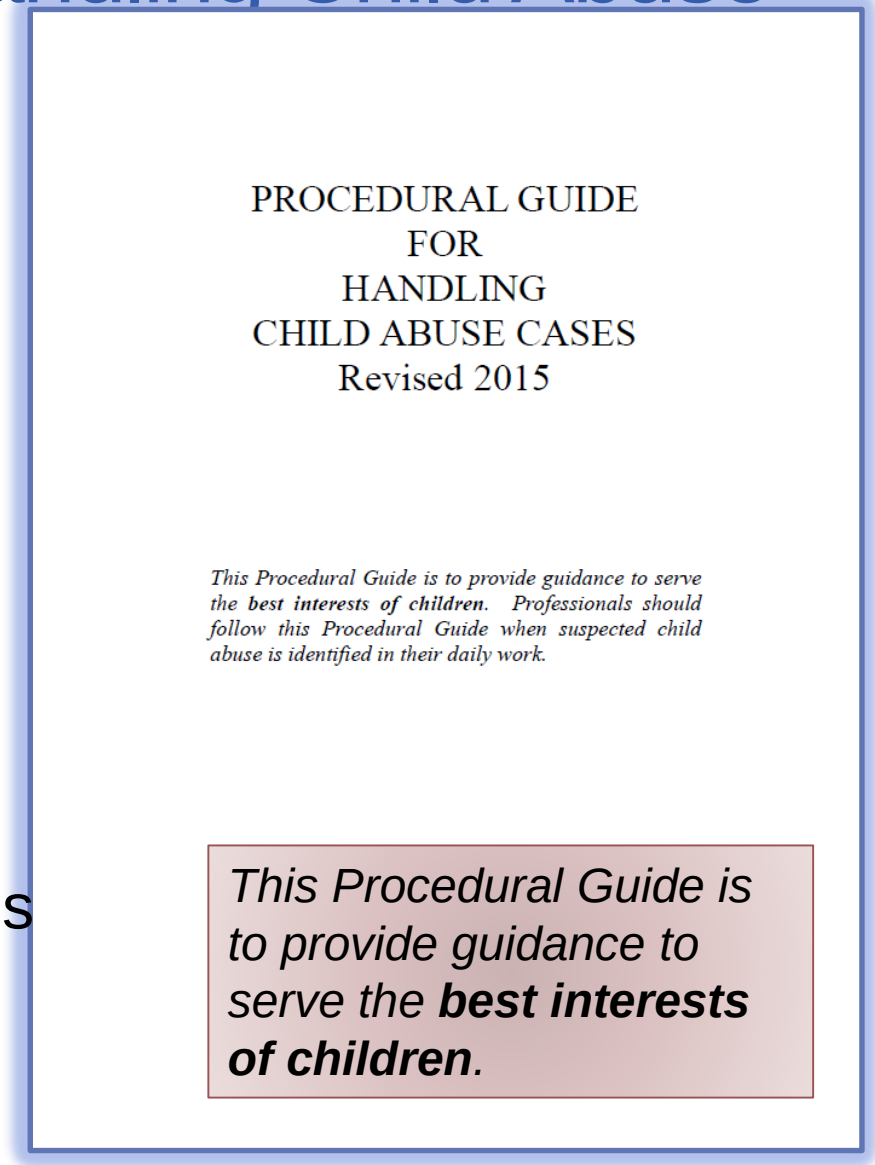
Child Abuse Investigation Unit (CAIU), CID

Handling suspected child abuse

- Social enquiry/ investigations
- Medical assessment and treatment
- Joint investigations (school, police)
- Multidisciplinary case conference
- Follow up +/- review meeting

Procedural Guide for Handling Child Abuse Cases

- Section I : Introduction
- Section II: Basic knowledge
- Section III: **Multi-disciplinary collaboration**
- Section IV: Handling of referrals/ Enquiry/ Investigation
- Section V: Roles of Relevant Departments/Organisations
- Section VI: Handling of Allegations against staff



PROCEDURAL GUIDE
FOR
HANDLING
CHILD ABUSE CASES
Revised 2015

*This Procedural Guide is to provide guidance to serve the **best interests of children**. Professionals should follow this Procedural Guide when suspected child abuse is identified in their daily work.*

*This Procedural Guide is to provide guidance to serve the **best interests of children**.*

Social Welfare Department, HKSAR

Multi-disciplinary collaboration Governing Principles

- Protecting a child from abuse is the **joint responsibility of different professionals** who may come into contact with the child.
- Priority should be given to **ensuring immediate safety of the child**.
- While it is appreciated that different professionals may have different views at different stages of intervention**, consensus** should be reached among the professionals as far as practicable, bearing in mind that the **safety and welfare of the child** should be the paramount concern.
- Making reference to the guidelines set out in Chapter 4 "Information Sharing and Confidentiality", **relevant information** about the child and / or the child's family should **be shared** among professionals involved in the helping process **on a need-to-know basis**.
- The **child should not be required to repeat the abuse incident(s**) except when necessary.

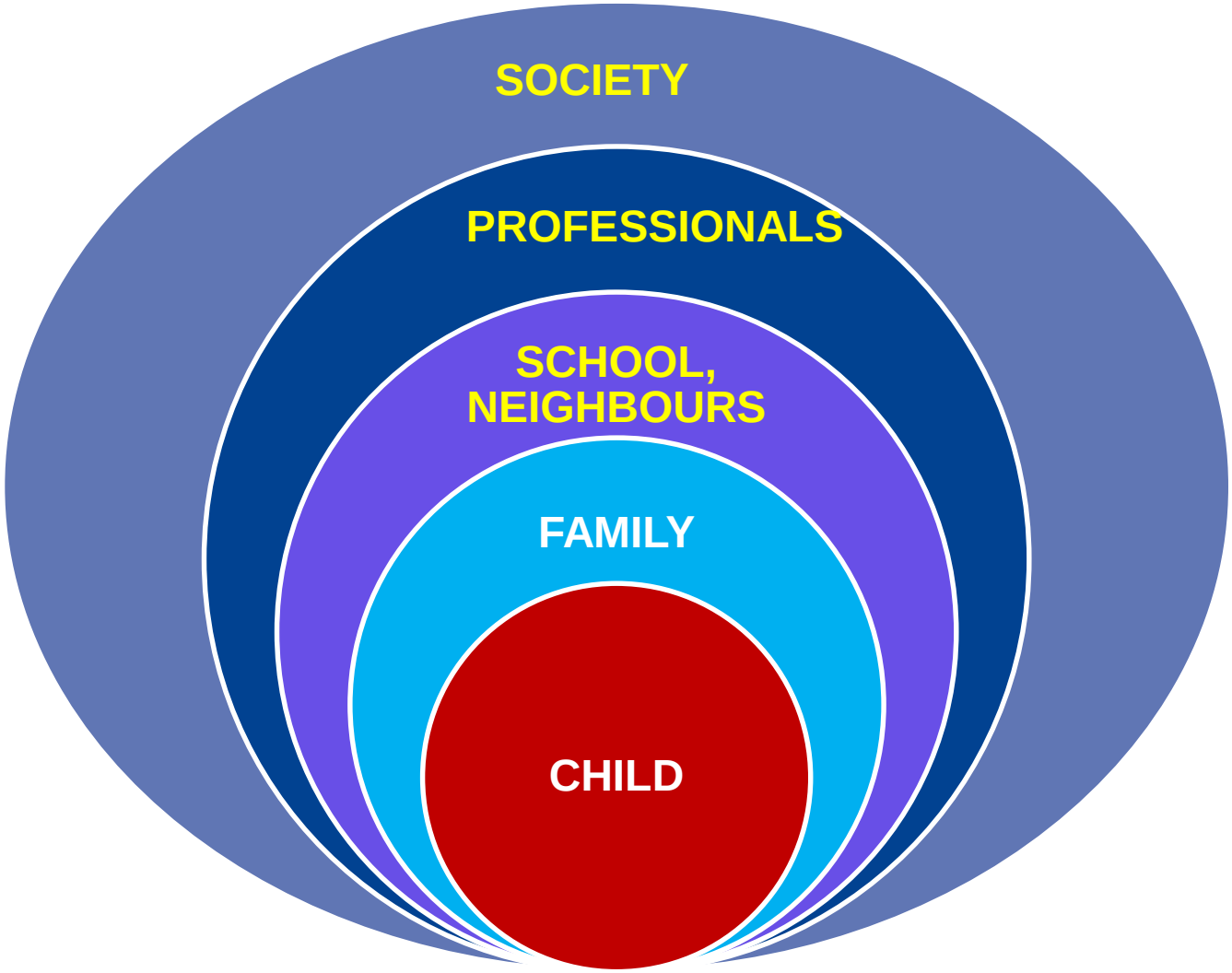
Hospital management

- Medical assessment and treatment
- Explain to parents/guardians the management plan.
- Strategic planning meeting – preliminary plan
  - Report to police (some cases require video-recorded interview)
  - Safety issues : other siblings
  - Home or placement
- **Multi-disciplinary case conference (MDCC)**
- **Communicate** with all stake-holders before the MDCC

Multi-disciplinary case conference (MDCC) - focus on child welfare

- Share information
- Case nature : abuse, not abuse, high risk
- Abuse/high risk – register on Child Protection Registry
- Care or Protection Order needed?
- Welfare plan
  - Placement: home, relatives, emergency foster homes, foster homes, small group homes, children's home, residential schools
  - Support services : psychological, family & school support, drug services
  - Follow up
  - Review meeting
  - Progress report
- Family participation

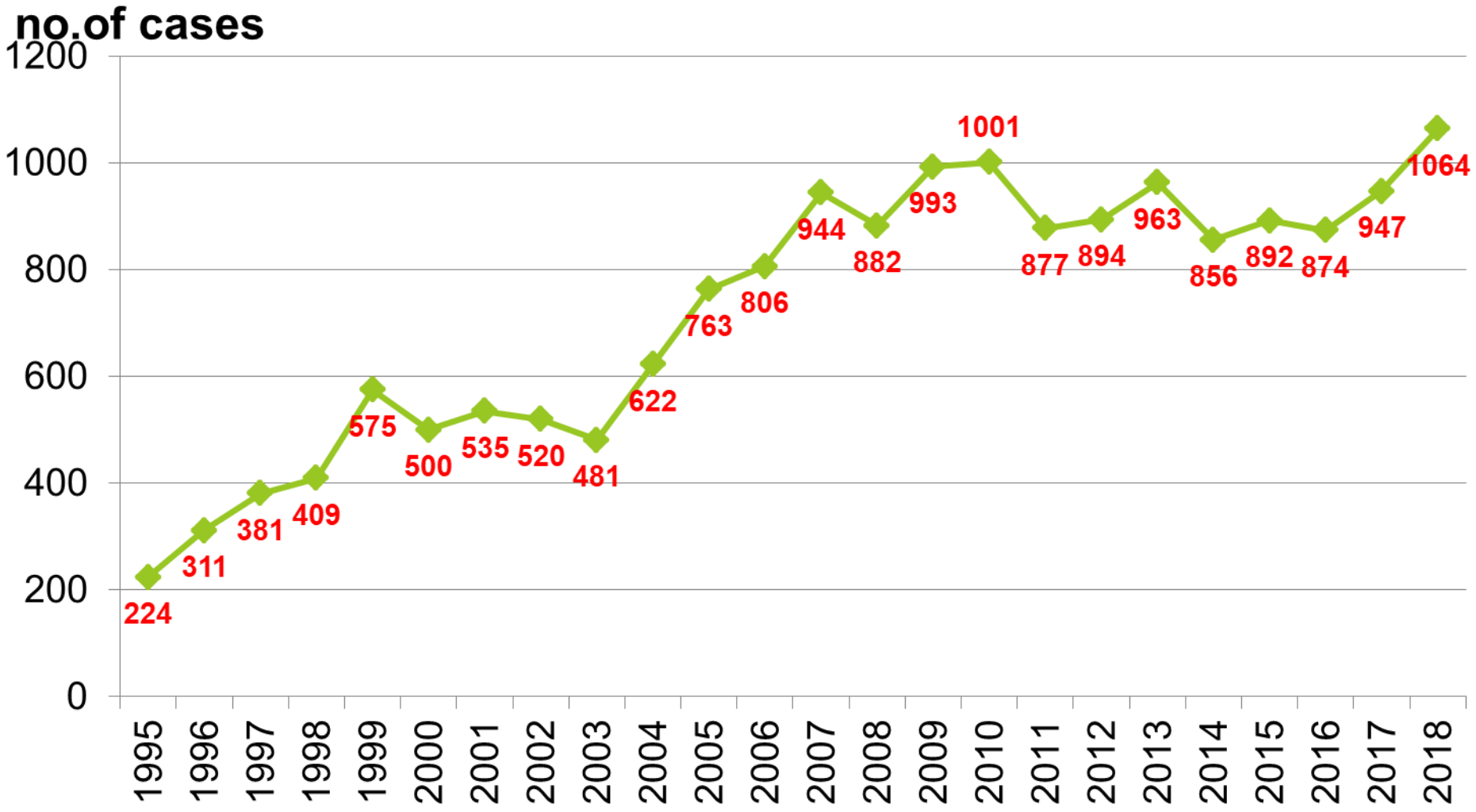
Working together to safeguard children



CHILD ABUSE IN HK

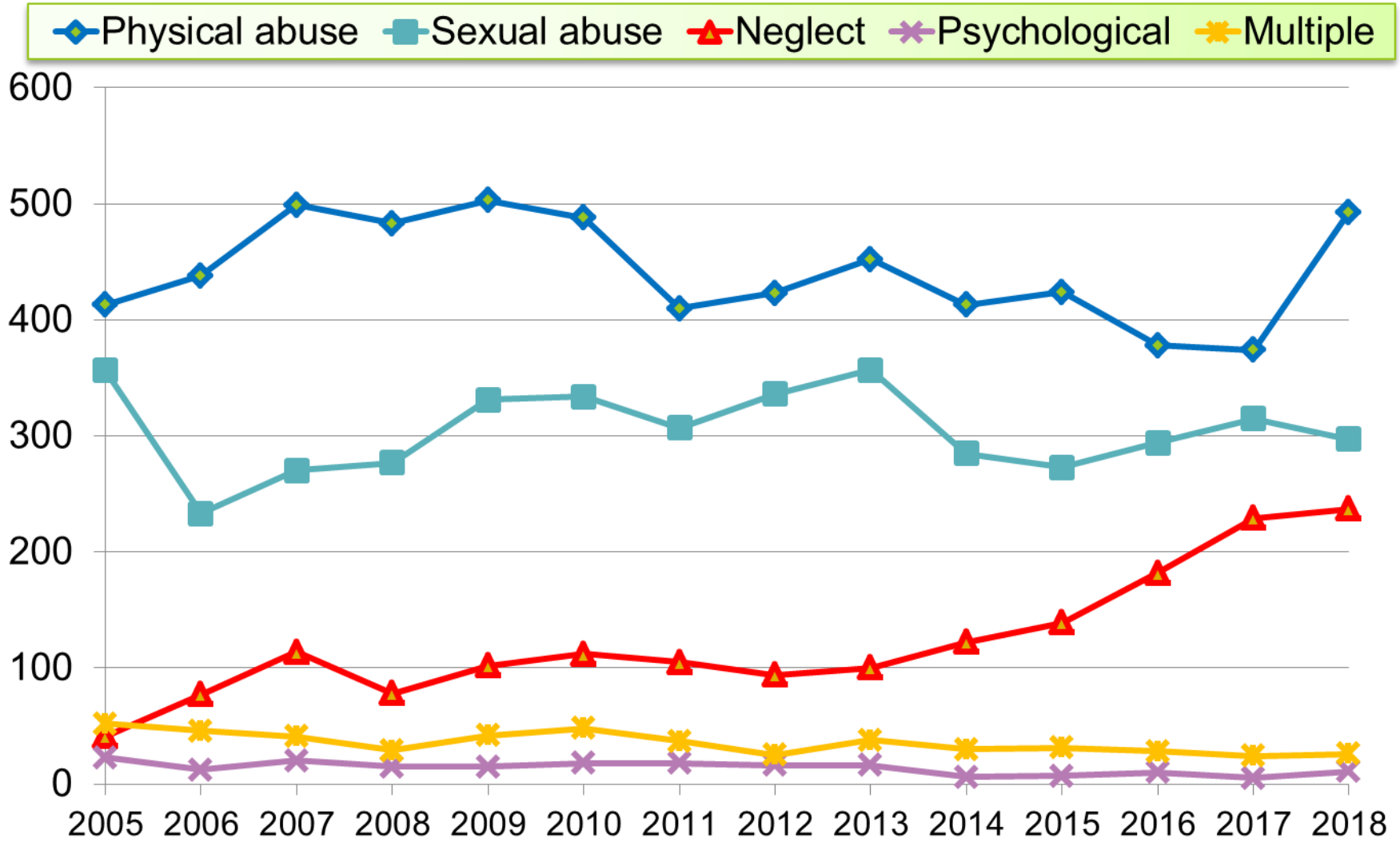
Source: Child Protection Registry
http://www.swd.gov.hk/vs/chinese/stat.html

Newly reported Cases of Child Abuse in HK



Source: Child Protection Registry, SWD
http://www.swd.gov.hk/vs/chinese/stat.html

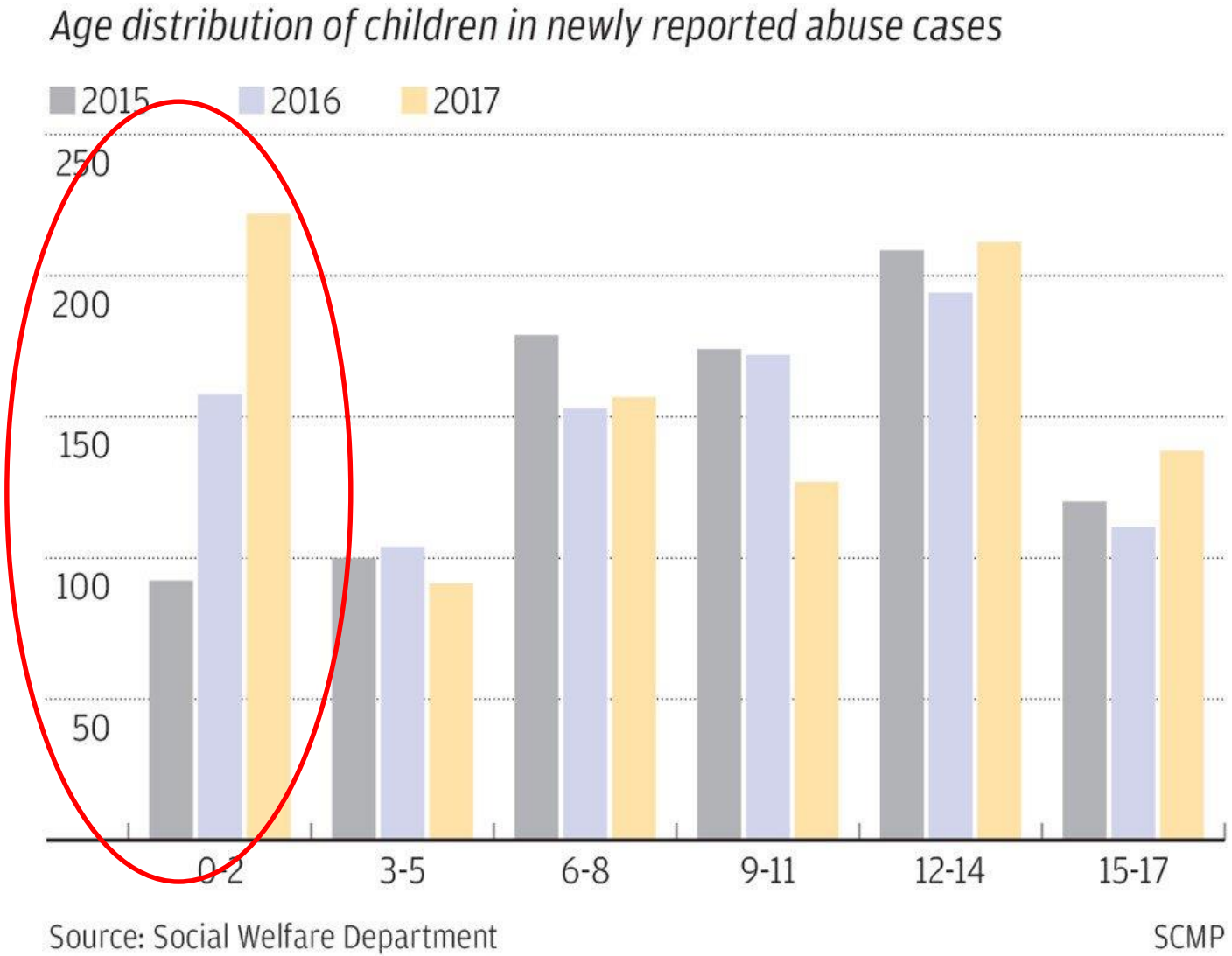
Newly reported Cases of Child Abuse in HK



Source: Child Protection Registry, SWD

http://www.swd.gov.hk/vs/chinese/stat.html

Child abuse in under 2 years old soars – 75% were neglect



Who are the abusers: ~65% are parents

Abuser's Relationship with the Victims	January to December 2018
	No. of Cases (%)
Parent	684 (64.5%)
Sibling	14 (1.3%)
Step-parent	42 (4.0%)
Grandparent	24 (2.3%)
Relative	12 (1.1%)
Family friend / Parent of peer **	51 (4.8%)
Caregiver	19 (1.8%)
School teacher / personnel	14 (1.3%)
Staff of boarding section of school **	3 (0.3%)
Tutor / Coach	24 (2.3%)
Religious personnel **	0 (0.0%)
Schoolmate / friend / peer **	43 (4.1%)
Co-tenant / Neighbour	7 (0.7%)
Inmate of residential service **	4 (0.4%)
Unrelated person (including strangers)	79 (7.5%)
Unidentified person	36 (3.4%)
Other	4 (0.4%)
Total *	**1 060 (100%)**

Where?

District of Incident Happened	January to December 2018
	No. of Cases (%)
Central & Western	21 (2.0%)
Southern	28 (2.6%)
Islands	21 (2.0%)
Eastern	53 (5.0%)
Wan Chai	12 (1.1%)
Kowloon City	39 (3.7%)
Yau Tsim Mong	54 (5.1%)
Sham Shui Po	51 (4.8%)
Wong Tai Sin	65 (6.1%)
Sai Kung	50 (4.7%)
Kwun Tong	126 (11.8%)
Shatin	36 (3.4%)
Tai Po	44 (4.1%)
North	61 (5.7%)
Yuen Long	127 (11.9%)
Tsuen Wan	33 (3.1%)
Kwai Tsing	82 (7.7%)
Tuen Mun	84 (7.9%)
Outside Hong Kong	37 (3.5%)
Unknown	40 (3.8%)
Total	**1 064 (100%)**

PREVENTION OF CHILD ABUSE

Prevention

- Primary/universal
  - educate the entire community to create social change that is intolerant of child maltreatment
    - E.g.mass media, family and child health services, family life education
- Secondary/selected
  - includes strategies that are focused on those who are at risk for abuse or neglect of their children
    - e.g.intervention program focused on transition to parenthood
- Tertiary/Indicated
  - encompasses treatment for families who already have encountered child abuse or neglect and serve to prevent the reoccurrence of such maltreatment.
    - E.g. family support program, family group conferencing, parenting skill classes, mass media to promote positive impact of intervention

High risk families 高危家庭

HA 医管局
PA&M
Psych

全面評估 Full assessment of family with management plan & tracking by O&G, Paed, Psy & MSW/NGOs

藥物濫用
精神病
少年懷孕
家庭暴力

Health risks
- **Substance abuse**
- **Mental illness**
- **Teen pregnancy**
- **Domestic violence**

母嬰健康院
M
C
H
C
- Parenting
- Immunization
- Growth & Develop Surveillance

1 CPaed

CPsy Service
- Growth & dev. problems
- Child abuse /Dom Violence
- Disabilities
- Behavioural problems

NGOs志願機構
SARDA 戎毒會
PS33軟性毒品
New Life 新生會

CAC
兒童智能評估中心

Treatment-oriented

Family Counselling Unit

Supporting families at risk (practical, emotional & training support)

Family Support Un

Open, development & preventive

Family Resource Unit

EETC
ICCC
SCCC
NGOs
早期及特殊幼兒中心

IFSC綜合家庭服務中心

(Courtesy of Dr P Ip)

Role of medical practitioner

- Be familiar with the common manifestations of child abuse
- Be motivated to report findings
- Keep a good medical record
- Be prepared to testify
- Prevention

Tragic death of a 5 year-old Down's syndrome boy

- On March 23, 2013, Yeung Chi-wai **died of methamphetamine (or ice) overdose**
  - with a toxic level 7X higher than the usual ice-related death
  - toxicologist suspected Chi-wai swallowed an entire piece of ice crystal which he probably picked up somewhere at home.
  - The single mother and her boy friend are known drug addicts
- A month before his tragic death, Chi-wai was found to have
  - multiple wounds on his body and was frequently absent from school
- A Multi-Disciplinary Case Conference (MDCC) concluded:
  - mother was unfit and incapable of taking care of her child
  - to put Chi-wai in a residential child care facility.
- However, MDCC finally allowed the child to be returned to his mother because there was no residential care service available at the time.

Learning objectives

- Recognize the different types of child abuse & their physical signs
- Differentiate between accidental and non-accidental injuries
- Understand the long term adverse effect of child abuse and the importance of child protection
- Understand the multidisciplinary approach to management

References

- American Academy of Pediatrics. Clinical Report. Evaluation of suspected physical abuse. Pediatrics 2015;135:5:e1337-1354.
- When to suspect child maltreatment . http://guidance.nice.org.uk/CG89
- Gilbert R et al. Recognising and responding to child maltreatment. Lancet 2009;373: 167-180.
- MacMillan HL et al. Interventions to prevent child maltreatment and associated impairment. Lancet 2009;373:250-266
- Reading R et al. Promotion of children's rights and prevention of child maltreatment. Lancet 2009; 373:332-343